

Improving Learning

Review of Written (CEQ & MEQ) Examinations, 2023

Final Report

**Submitted to the Royal Australian and New Zealand
College of Psychiatrists (RANZCP)**

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EXECUTIVE SUMMARY

BACKGROUND AND CONTEXT

The Royal Australian and New Zealand College of Psychiatry (RANZCP) Board recently commissioned the Australian Council for Educational Research (ACER) to review the Critical Essay Question (CEQ) examination. Psychiatry trainees are required to complete the CEQ, a 50-minute, handwritten essay responding to a quote, as part of the summative examinations for the RANZCP. The ACER review centres on considering the CEQ's effectiveness, fitness-for-purpose and ongoing role in the RANZCP assessment program.

The intended purpose of the CEQ, as specified by the College, is to assess written, analytical, integrative, critical and reflective thinking skills. It aims to achieve this by requiring candidates to respond to a quote that can be related to a fundamental (and typically complex) topic from the history and philosophy of psychiatry. The CEQ intends to draw on candidates' understanding of how different factors have influenced the trajectory of psychiatry as a discipline, their appreciation of the importance of cultural contexts, and their ethical considerations. There is significant debate in the literature on the definition of critical thinking, how (and if) it should be taught, and specific measures that might provide an assessment of students' critical thinking skills. Recognising that the CEQ may not be assessing these important skills effectively, RANZCP was interested in exploring other assessment formats. There may be alternative assessment formats that can offer a more holistic picture of the candidate's analytical, integrative, critical and reflective thinking skills, which may even be non-summative. This move aligns with shifts in contemporary medical specialist training and assessment, where varied assessment formats are utilised to provide longitudinal assessment data and accumulated evidence of learning, with more integration with work-based training and as part of a whole program of assessments.

PURPOSE AND OBJECTIVES OF THE REVIEW

ACER were asked to first evaluate the internal stakeholder consultation findings undertaken by the RANZCP about the CEQ. A desktop review of the existing CEQ material and of the RANZCP curriculum learning outcomes and learning activities focused on the development of these skills informed an articulation of the content and skills assessed by the current CEQ format. The project was also informed by contemporary directions for assessing such skills in comparable psychiatry specialist medical contexts. After completing this preliminary work, RANZCP requested that ACER conduct its own consultations with stakeholder representatives, to validate the findings from the internal consultations and seek further independent information and evidence around the CEQ. ACER also explored other relevant alternative assessment formats that could assess the CEQ construct.

RANZCP also sought guidance from ACER in exploring approaches to improving the quality of the Modified Essay Question (MEQ) examination. As the MEQ component of the project was quite separate from the CEQ review, the MEQ review is presented as a comprehensive Appendix.

METHODOLOGY

The project was split into two main Phases, as per the key activities and timelines outlined in the Project Plan. Phase 1 involved project initiation, a desktop review and preliminary evaluation. Phase 2 involved consultations undertaken by ACER with current trainees and Fellows of the College and development of this Final Report. ACER provided an updated Project Plan and Briefing Paper for the RANZCP Board (on May 23); delivered an Interim Report after Phase 1 (May 31) and a Final Report after Phase 2 (late August).

The findings of the assessment reviews undertaken by ACER are underpinned by several key concepts and principles drawn from the contemporary literature on assessment best practice, as well as contemporary theories of assessment practice relevant to medical education and speciality training contexts. This project was guided by this evaluative framework.

SUMMARY OF FINDINGS FROM CEQ DESKTOP REVIEW

Evaluating the RANZCP stakeholder consultations

- Overall, stakeholder views of the original CEQ and their preferences for potential alternate assessment formats varied significantly. Stakeholders clearly prioritised different assessment aspects (e.g. reliability, validity, acceptability, feasibility) in their preferences for alternate assessment formats.
- There is limited support for retaining the CEQ in its current format; only the Committee for Examinations (CfE) overwhelmingly support this option.
- Although stakeholder perspectives varied, the need to carefully review and define the critical thinking construct was widely endorsed. There was a strong desire to review the place of the CEQ in the overall program of assessment, to clearly define the skills the CEQ is designed to assess, and to identify how these are taught within the training program.

The utility of the CEQ

- Assessment is an optimisation problem. No one assessment method is perfect in every aspect, every assessment method has strengths and weaknesses, and the utility of any assessment approach depends on the purpose of the assessment process. The utility of the CEQ was evaluated against the five criteria of validity, reliability, educational impact, acceptability and cost/feasibility. The first three are particularly problematic for the CEQ.
- The validity of the CEQ is questionable, particularly due to inherent limitations of the essay format that rewards specialised writing skills, but also because no other forms of validity evidence were supportive of the claim that the CEQ measures what it is meant to be measuring. The CEQ becomes artificially more challenging for Specialist International Medical Graduates (SIMGs) and/or candidates where English is not their first language. These candidates may be able to demonstrate proficiency in the relevant skills in a different format, which undermines the potential validity of the current CEQ. It is also unclear whether the specialised essay-writing skills assessed by the CEQ align with those required in psychiatric practice. The relevance to psychiatric practice of being able to write a sophisticated and nuanced essay on a specific quote under time pressure is doubtful.
- As the CEQ comprises only one question, utilising a single quote, there are major reliability issues with the CEQ. Reliability is generally enhanced with a longer assessment and more questions.

- The educational impact of the CEQ is also problematic. Candidates with a strong command of written English and well-developed essay writing skills could do well in the CEQ with little preparation, and thus won't necessarily learn much from the studying process. Conversely, candidates who believe they will struggle with the CEQ will likely learn to write better essays in preparing for the CEQ. However, this does not necessarily mean that they will improve their critical thinking skills. Instead, they are likely to learn techniques and strategies in written communication needed to pass the CEQ.

Articulating the CEQ construct

- To clearly articulate the CEQ construct, we began by reviewing the blueprinting (mapping) between the Fellowship Competencies and the CEQ marking guide. The Fellowship Competencies are aligned to the CanMEDs Framework but are rather generic.
- The College should be commended for attempting to assess these non-medical expert domains summatively. However, the mapping of the RANZCP Fellowship Competencies to the CEQ feels somewhat forced, which is demonstrated by the lack of alignment in the proficiency level descriptions. It is plausible that people with critical thinking, creative and essay writing skills could perform well in the CEQ even if they are not in the RANZCP training program (or indeed, even if they are not medically trained).
- We articulated two constructs – the first defined what the CEQ is currently assessing, and the second defined what the CEQ should (or could) be assessing. The first articulation (what the CEQ is currently assessing) feels overly constrained by the need to demonstrate proficiency in essay writing skills.
- The rearticulated CEQ construct we provided centres on *thinking*. It is likely that proficient candidates would readily recognise multiple perspectives on an issue, appreciate drivers of perspectives, recognise potential tensions, and examine contentious aspects of an issue. And of course, a proficient candidate would likely understand the issues in a range of contexts; however, this need not be communicated in written format.

Alignment to the RANZCP curriculum

- It is evident that the skills required to demonstrate critical thinking through a critical and nuanced essay are not being taught as part of the training program.
- Upon evaluation of the Learning Outcomes of Stage 2 of the program, it was clear that none of these explicitly related to the acquisition of critical thinking skills as assessed through the CEQ.
- We thus see a gap between the skills currently being assessed in the CEQ format and how these skills map/align to learning outcomes in the training program. Furthermore, we did not identify any structured learning activities or *ad hoc* learning opportunities for trainees or SIMGs that could represent learning moments for the skills assessed in the CEQ.

SUMMARY OF FINDINGS FROM ACER INTERVIEWS

- The consultations undertaken by ACER with eighteen RANZCP stakeholders align with the variable perspectives on the CEQ identified during RANZCP's own internal consultations.
- Although there is some limited support for retaining the CEQ unchanged, the majority of stakeholders consulted strongly support better articulating the purpose of the CEQ, linking the purpose of the CEQ more strongly to the training program outcomes, providing better teaching and learning opportunities for trainees to develop the skills thought to underlie the

CEQ, and considering alternate assessment formats to assess the relevant construct that link better to psychiatric practice.

SUMMARY OF FINDINGS RELATING TO ALTERNATIVE FORMATS

International Comparisons

- To identify assessment formats that might compare with the intention and format of the CEQ, we briefly reviewed the assessment programs of the Royal College of Psychiatrists in the UK and the Royal College of Physicians and Surgeons in Canada for the psychiatry specialty.
- A brief examination of these curricula and examinations did not identify any standalone written assessment format analogous to the CEQ. Aspects of both these curricula resonate with themes of complexity and uncertainty, understanding of the broader historical context of psychiatry and of the multitude of factors that might influence psychiatric care. These learning outcomes are also well-connected to understanding their influence on the provision of care to patients and to integrating broader understanding to improving outcomes for patients.
- In these contexts, deeper exploration of trainees' understanding of the history of psychiatry and complex issues affecting patient care and outcomes appear to be assessed through a combination of workplace-based assessment and oral assessment related to patient scenarios (rather than in a standalone format or disconnected from clinical cases).

Considerations from Programmatic Assessment

- The medical education literature has seen the steady rise over the past fifteen years of an approach to assessment known as 'programmatic assessment'. A programmatic approach to assessment treats assessment as a whole program, rather than as isolated or ad hoc assessment instruments
- A programmatic perspective on the CEQ would instead judge the strength and value of the RANZCP assessment program by the quality and implementation of the programme as a whole. In programmatic assessment, a variety of well-chosen and well-developed assessment tools, both written and workplace-based, are implemented in low, medium and high stakes contexts as appropriate.
- In programmatic assessment, the distinction between training and assessment is deliberately blurred. Some tasks may focus more on their learning function, but still generate assessment data for review from a low-stakes context, while other tasks may focus more on a certification or hurdle function – where the purpose of the assessment is to demonstrate mastery in a medium or a high-stakes context.
- Such a program of assessment thus provides adequate sampling across the breadth and depth of the curriculum, as well as maintaining a focus on the value of meaningful feedback alongside the necessary certification function of assessment systems. There are multiple opportunities for gaps in learning to be identified, and when combined with an assessment culture which recognises and supports the important formative role of assessment information, allows remedial work to be instigated early and supportively. A programmatic approach should mean that a summative judgment of trainee underperformance should not come as a surprise to either the candidate or the supervisor of training.

Proposed Alternatives

- Drawing on best practice in assessment design and with consideration to the findings of this review, our recommendation is to retire the existing CEQ. We encourage the College to strongly consider alternate formats for assessing the skills intended to be assessed by the

CEQ. We propose four possible alternatives below and briefly outline considerations related to feasibility and appropriateness in light of our review. Our suggestions are intended to stimulate further RANZCP consideration and discussion, and could be fully adopted, partially adopted or adopted with modifications (depending on preferences and the resources available). Following the discussion of the suggested alternatives to the CEQ, Table 1 is included which highlights a broader range of options. These further options include clear statements of their advantages and disadvantages to support why these options are not recommended in the RANZCP context.

- Drawing on the comments above related to a programmatic approach to assessment, we strongly recommend that in retiring the CEQ, the intended construct should become the focus of a range of reconstituted learning and assessment activities. This should not be interpreted as simply replacing the CEQ with a new examination. It should be thought of as a broader philosophical shift to both better assess this construct and better facilitate its learning acquisition – embracing this ‘dual purposing’ of programmatic assessment.
- The four options for consideration that we regard as feasible and acceptable alternatives to the CEQ are outlined below. The first three options propose some variation on a structured oral assessment (described in full from page 42) with structured learning activities (described in full from page 44), while the fourth option proposes only structured learning activities in conjunction with formal education.

Option 1: A structured, medium stakes, summative oral assessment task flexibly delivered in the workplace (external examiner assessed at a time the trainee chooses online) in conjunction with structured learning activities in the workplace to develop candidates’ knowledge, skills and abilities related to the CEQ construct. Consideration might be given to assessing written skills as part of selection to the training program for this option.

Option 2: A take-home written essay followed by a structured, medium stakes, summative oral assessment task flexibly delivered in the workplace (external examiner assessed at a time the trainee chooses online) to explore candidates’ thinking in the context of their written submission in conjunction with structured learning activities in the workplace to develop candidates’ knowledge, skills and abilities related to the CEQ construct.

Option 3: A structured, high stakes, summative oral assessment task delivered centrally in conjunction with structured learning activities in the workplace to develop candidates’ knowledge, skills and abilities related to the CEQ construct. Consideration might be given to assessing written skills as part of selection to the training program for this option.

Option 4: Structured learning activities in the workplace to develop candidates’ knowledge, skills and abilities related to the CEQ construct and completion during the training program of compulsory courses in the history and philosophy of psychiatry (either undertaken as part of formal education programs or developed and delivered centrally by the College).

- Each of Options 1-3 introduces a new structured oral assessment task to assess nuanced thinking that embraces multiple perspectives in a psychiatric context. A bespoke name and acronym could be developed, but for simplicity, we will refer to this task as a Structured Oral Assessment (SOA).
- The SOA format involves interaction between the candidate and the assessor. We see this interaction as a strength of the proposed SOA format, where responses can be clarified and

probed in real-time, and a dialogue can be facilitated around a topic. In this way, the significant limitations of the written format and the challenge to fairness can be overcome.

- We deliberately downplay comparisons to a high-stakes OSCE, and instead present the SOA as similar to a medium-stakes Case-Based Discussion (CBD).
- For Options 1-3, we suggest the implementation of the SOA should remain summative as a hurdle requirement. We acknowledge that assessment drives learning, and higher-stakes assessments need to be aligned with the intended outcomes of a training program. Maintaining a summative implementation will encourage trainees/SIMGs to engage more seriously with the relevant content than they may otherwise if the assessment were purely a workplace-based task.
- Options 1 and 2 for the SOA could be implemented flexibly in the workplace with external assessors trained to provide feedback and assessment against the marking criteria. We suggest that the timing of the SOA for Options 1 and 2 could be relatively flexible and scheduled by trainees during set periods of Stage 3 of the training program when they believe they are prepared.
- Option 2 gives consideration to the inclusion of a written submission completed by the candidate in their own time should the College wish to maintain a written component within the examinations program. Candidates could submit a written piece in response to a prompt from a topic list. This could take the form of an essay but allow the candidate some research and writing time. The piece could encourage reflection from the candidate on the issue at hand, and in certain respects, could mirror the existing CEQ.
- An oral defence of the submission under Option 2 would facilitate a summative assessment of written communication but would have the added benefit of minimising the risk of candidate cheating. In the context of the recent proliferation of generative artificial intelligence (AI) tools, such as ChatGPT, the College should assume that candidates will likely use all the tools available to them to prepare a submission. It should be made clear to candidates that AI tools can be used as a support and learning tool but the work must be their own, and if the assessors are concerned about the authenticity of the work (especially after the oral defence), they will be marked down.
- As wholly oral assessments, Options 1 and 3 include consideration of the assessment of written skills at the time of selection to the program.
- The success and efficacy of these alternative assessment options would depend upon a dedicated panel of assessors who were committed to the intent and purpose of the new format and were prepared to both develop cases with marking rubrics and deliver them periodically to candidates. Capitalising on the interest and expertise of the current CEQ team to champion this new format, drive the quality development of the content, and manage the examination (for Option 3) would be valuable. Management of the existing CEQ until it was retired would remain with the current CEQ team.
- Each of Options 1-4 also include a series of new oral format Structured Learning Activities (SLAs). During our consultations it was clear that the opportunities for trainees to access teaching and learning opportunities for the skills underlying the CEQ during the training program were insufficient, variable or absent. Stakeholders identified a lack of formal teaching (such as classes on ethics and the history of psychiatry), the insufficiency of teaching resources provided by the College on the CEQ format, and variable learning opportunities in the workplace that often depended on individual supervisors.

- We thus propose a series of SLAs relating to the construct in question, to better facilitate the acquisition of these skills during training. While the proposed SOAs in Options 1-3 remain a summative hurdle, developed centrally by a dedicated assessment committee to maintain performance standards, standardisation and oversight, the SLAs can in practice become less formal and less standardised to maximise their learning function. However, we call these 'corresponding' SLAs because the assessment format can in fact be identical to the summative SOAs. That is, the SLAs will function as practice tasks to encourage the development of candidates' thinking and help trainees and SIMGs prepare for their hurdle requirement.
- Option 4 is the only one of the proposed options not to include a medium-high stakes SOA. Instead, this option includes SLAs as participation hurdle tasks in conjunction with additional formal education on the history and philosophy of psychiatry (either through College-developed education or other external formal education programs). It is possible that without the motivation of the medium stakes oral assessment, trainees' engagement with wider reading related to the CEQ construct may diminish.

Table 1. Proposed options for replacing the CEQ assessment format and their advantages and disadvantages.

Option	Advantages	Disadvantages	Recommended
1. Structured Oral Assessment (SOA) in conjunction with Structured Learning Activities (SLAs) delivered in the workplace	Greater fairness, acceptability and validity of an oral format; specific learning activities in the same format as the summative oral provide for more consistent learning opportunities for trainees and increased fairness. Summative format will drive trainee engagement and learning. Delivery in the workplace or online provides greater flexibility and reduced stress for trainees and reduces central examination burden. Options for supervisor and/or external assessors. Multiple options for feedback and learning aligns with a programmatic assessment approach.	Perceived burden of additional workplace-based assessment (offset by options for external assessors). No written component.	Yes
2. Written essay completed in candidate's own time followed by Structured Oral Assessment (SOA) in conjunction with Structured Learning Activities (SLA) delivered in the workplace	Greater fairness, acceptability and validity of an oral format; maintains a written component but with an oral defence to mitigate against cheating. Specific learning activities in the same format as the summative oral provide for more consistent learning opportunities for trainees and increased fairness. Summative format will drive trainee engagement and learning. Delivery in the workplace or online provides greater flexibility and reduced stress for trainees and reduces central examination burden. Options for supervisor and/or external assessors. Multiple options for feedback and learning aligns with a programmatic assessment approach.	Perceived burden of additional workplace-based assessment (offset by options for external assessors). Authenticity concerns due to the proliferation of AI tools.	Yes

3. Structured Oral Assessment (SOA) delivered centrally in conjunction with Structured Learning Activities (SLAs)	Maintains a high-stakes-summative focus but with the increased fairness, acceptability and validity of an oral format. Summative format will drive trainee engagement and learning. Specific learning activities in the same format as the summative oral provide for more consistent learning opportunities for trainees and increased fairness. Multiple options for feedback and learning aligns with a programmatic assessment approach. No additional workplace-based assessment component.	Resources required to deliver a central examination. No written component.	Yes (less preferable)
4. Structured Learning Activities (SLA) in conjunction with formal education courses on the history and philosophy of psychiatry	Specific learning activities and formal education provide for more consistent and fairer learning opportunities for trainees. Fewer resources required as there is no summative assessment component. No additional burden on supervisors. Less stress for trainees as there is no summative assessment component.	The lack of a summative assessment component related to the SLAs may reduce trainees' engagement with learning. Resources required to develop formal education courses (if delivered by the College).	Yes (less preferable)
5. Written essay completed in candidate's own time followed by Structured Oral Assessment (SOA) delivered centrally.	Greater fairness, acceptability and validity of an oral format; maintains a written component but with an oral defence. Summative format will drive trainee engagement and learning. No additional requirements for supervisors in the workplace.	No learning opportunities for trainees to support preparation for the SOA reduces fairness and acceptability. Resources required to both assess a written essay and deliver a central examination. Less flexible for trainees in exam timing and more stressful.	No
6. Written essay completed in candidates' own time	Fewer resources required as there is no summative assessment component. No additional burden on supervisors. Less stress for trainees as there is no summative assessment component.	Difficulties in establishing the written work as the candidate's own. Maintaining the essay format similarly retains the same challenges as the current CEQ format. No learning opportunities for trainees to support preparation for the CEQ further reduces fairness and acceptability. The	No

		lack of a summative assessment component may reduce trainees' engagement with learning.	
7. Retain the CEQ unchanged	No additional resources	The current CEQ has questionable reliability and validity and unacceptable fairness, acceptability and relevance. No mandated formal learning opportunities for trainees to support preparation for the CEQ further reduces fairness and acceptability.	No
8. Lengthen the MEQ to include vignette-based questions that target the CEQ construct	High-stakes examination already being delivered by the College.	As the MEQ assesses a different construct, this option would be analogous to running two exams in one. The examination would need to be significantly longer to ensure reliability and validity of the CEQ targeted questions, increasing the burden and stress for candidates. No learning opportunities for trainees to support preparation for the CEQ further reduces fairness and acceptability. Maintaining the written format for the CEQ construct reduces fairness and acceptability to candidates.	No

SUMMARY OF FINDINGS FROM MEQ DESKTOP REVIEW

- MEQs are generally clear and unambiguous, with marking rubrics that mostly align to the questions. There is also clear blueprinting to topics, age brackets and the curriculum.
- Further guidance could be given to candidates by stating the number of diagnoses/treatments/management practices that need to be discussed to achieve full marks. More detail could also be given to examiners on the breakdown of marks.
- Marking processes appear to be working well, and calibration after the marking of 5-7 scripts follows best-practice for essay-style questions. Each sub-part question should be carefully considered. If there is a significant risk that some sub-part answers will overlap, the same examiners should be used to mark those sub-part questions.
- Standard setting using both the CfE and Satellite group is useful for obtaining a cut-score from multiple perspectives. Error adjustments are applied to the passing standard and impact analysis is performed after the examination, which follows best-practice.
- The examination reports include extensive detail, advice to candidates and the release of one of the MEQs with its marking guide. The letters provide detailed feedback to candidates, including a breakdown of how they performed in different content areas. The written congress slides are also informative, and give an excellent overview of the MEQ development, standard setting and final result process.

COMMENDATIONS

We would like to make several commendations in relation to the intent of the CEQ and the commitment of all stakeholders to the RANZCP assessment program.

- The construct that the CEQ is designed to assess appears to add value to psychiatric training. Despite our finding that the CEQ in its current form is not fit-for-purpose, we see the value in its intention, especially with respect to the attempt to assess the non-medical expert Fellowship Competencies in a summative fashion (which in turn drives candidate learning).
- Despite the varied perspectives throughout this review in relation to the CEQ, all stakeholders were highly engaged in providing feedback about this form of assessment. In this sense, there was a clear and strong commitment to improving psychiatric training and a desire to continually improve the assessment of learners.

RECOMMENDATIONS

Recommendations relating to the CEQ

1. Consider the range of findings outlined in this report, and specifically explore the acceptability and feasibility of our proposed SOA/SLA options.

- Seek feedback on the findings and recommendations of this report from relevant RANZCP stakeholders, provide a forum for clarifying issues, and work towards building consensus.
- Seek feedback from stakeholders on the four options proposed by ACER as possible alternatives to the current CEQ that comprise different combinations of Structured Learning Activities (SLAs) and a hurdle Structured Oral Assessment (SOA).
- Consider the strengths and weaknesses of the alternative implementation options.
- We recognise that each of the proposed options may include aspects that are challenging to operationalise due to a range of factors. Thus, for any preferred option a partial or staged

implementation should be considered (for instance perhaps rolling out the SLAs in advance of any change to the summative assessment component)

2. Capitalise on the existing CEQ committee to lead a new SOA/SLA Committee tasked with conceptualising, developing and implementing the new suite of SOA/SLA assessment/learning activities.

- Engage existing CEQ examiners as part of this process and seek to leverage their expertise in reforming this new suite of assessment/learning activities.

3. Ensure these new SLA/SOA tasks are clearly linked to the curriculum and make sure the learning outcomes being assessed are relevant and clear.

- There is a current gap between the skills that are meant to be assessed in the CEQ construct and the articulation of these skills as Learning Outcomes in the training program.
- Once the new Learning Outcomes are finalised, ensure that the SOA/SLA solution adequately assesses these skills, and explicitly blueprint SOA/SLA tasks to specific Learning Outcomes.

4. Trial some initial draft SOA/SLAs to inform the finalisation of the assessment requirements.

- Trialling is an important part of the assessment development process. This can be done simply. For examples, once several drafts have been finalised, seek candidates to trial the cases/vignettes in controlled scenarios. The candidates could be psychiatrists who have recently gained RANZCP Fellowship, or even trainees and SIMGs who have recently passed the CEQ. Ask for feedback from participants on all aspects of the assessment process and collate the feedback for evaluation by the SOA/SLA Committee.

5. Finalise and publish the structure of the chosen SOA/SLA process.

- This should include clarity around timing of the assessment, number of cases/vignettes, passing requirements for the hurdle SOA, number and timing requirements of the SLAs, etc. These details need to be communicated prior to the implementation of the reform solution.
- The SOA/SLA Committee may decide to release new sets of vignettes every 6 months, or recommend specific topics, for instance. Details such as these should be worked-through and arrived at by the Committee.

6. Release a suite of learning resources and support material, including a list of cases/vignettes to support learners in their preparation for the SOA.

- The SOA/SLA committee will need to prepare and release a range of learning resources and sample cases/vignettes to provide support for candidates and supervisors. There will need to be a long list of topics (vetted by the College via the committee), with lists of suggested readings for the topics.

7. Consider establishing an Expert SOA Assessors Group.

- This group of external assessors could support trainees/SIMGs in their preparation and during the SLA and serve in a mentoring capacity to support the development of the relevant skills. Such assessors would be a step-removed from the supervisor context and may be more effective in providing constructive feedback. It would also reduce the burden on supervisors (particularly for Options 1 and 2 where SOAs are not centrally delivered) and draw on the collective expertise of a distributed group of experts who were interested in the SOAs.
- This group could also deliver the summative SOAs and take ownership of the assessment. Some may sit on the SOA/SLA Committee as well, but we envisage the need for a larger pool of centralised assessment experts to reduce the collective workload.

8. *Develop guidelines for SOA Assessors and Supervisors undertaking SLAs, and provide training.*

- The guidelines should outline the purpose of the assessment and learning activities and provide guidance on acceptable forms of prompting and candidate interaction. The SOA Assessors would also need to be briefed and trained periodically in appropriate conduct, and potentially engage in calibration sessions.

9. *Retire the CEQ and replace with the acceptable and feasible options relating to SOA/SLAs.*

- This should only be done once the SOA/SLA Committee has developed sufficient support material and it is feasible to transition to the new assessment/learning activities.

10. *Evaluate the SOA/SLA solution after its initial implementation and commit to continuous improvement of the process.*

- Routine evaluation is good practice, but in this case we strongly recommend evaluating the quality of the SOA/SLA model and the chosen implementation format after the initial roll-out and perhaps every 12 months. If the solution is only partially adopted, the evaluation should review the efficacy of the partial implementation and the need for further adaptations.

11. *Encourage trainees/SIMGs to engage deeply with the history and philosophy of psychiatry and the broad range of social, cultural, ethical and economic factors influencing psychiatric practice.*

- This could be implemented through a range of strategies (to supplement the learning that would occur during the SLAs), such as: developing College run courses in the history and philosophy of psychiatry; hosting lecture series and webinars by expert researchers; developing webinars to teach trainees/SIMGs; or providing book lists and library catalogues on an array of topics.

Recommendations relating to the MEQ

A1. *Move the MEQ to an online format.*

- In doing so, carefully consider the impacts of a digital proctored solution as well as the feasibility of using testing centres prior to finalising the delivery method. It may be worth exploring the feasibility of both options.

A2. *Update the standard setting terminology used in the MEQ marker recruitment and calibration Standard Operating Procedures.*

- The current version of the *MEQ marker recruitment and calibration Standard Operating Procedures* states that standard setting is performed using the Ebel methodology. However, the committees appear to be using an extended Angoff method where the score of the minimally competent candidate is estimated for each question sub-part. The Standard Operating Procedures should be updated to reflect the current standard setting process.

A3. *Consider providing advice on when to sit the MEQ.*

- The range of 18 months to 60 months is quite broad. It could be worth analysing the average number of months a candidate has been in the program when they successfully complete the MEQ and providing some advice to candidates related to this evidence.

1. INTRODUCTION

1.1 BACKGROUND AND CONTEXT

The Royal Australian and New Zealand College of Psychiatrists (RANZCP, 'the College') Fellowship examinations include two handwritten essay-style examinations: the Critical Essay Question (CEQ) examination and the Modified Essay Questions (MEQ) both of which are designed to assess a capacity for critical thinking as it applies to psychiatric practice. These examinations were previously delivered in conjunction; however, following the Australian Council for Educational Research (ACER) review of the Fellowship examinations in 2020, the RANZCP Board accepted ACER's recommendation to decouple the CEQ and MEQ and offer these as two independent examinations.

Significant concerns remain about the fitness-for-purpose of the CEQ. In particular, ACER's 2020 review identified the lack of an assessment framework clearly linking what the CEQ assessed to the learning outcomes of the broader training program. Thus, it was unclear how the skills being assessed by the CEQ under examination conditions reflected real-world psychiatric practice.

RANZCP has continued to gather evidence to inform future directions for the CEQ. The College developed a CEQ review paper exploring alternative assessment formats for critical thinking. RANZCP also recently conducted a comprehensive consultation process with relevant stakeholders and committees, including trainees, focused on responses to the CEQ review paper. The CEQ review paper along with the stakeholder feedback has been considered by the RANZCP Education Committee. This process showed that stakeholders' views regarding the value of the CEQ and the format in which critical thinking skills might be assessed varied significantly. Most stakeholder groups did not support retaining the CEQ in its current format for reasons such as a perceived lack of validity, or because removing the CEQ from the summative assessments for the RANZCP could reduce the perceived 'burden of assessment' without substantially altering the overall integrity of the examinations. Some stakeholders, however, maintained that the skills assessed by the CEQ are critical to psychiatric practice and argued that the CEQ should be retained in its current format. As part of this internal consultation process, RANZCP stakeholders also identified that the written English language skills required in the current format of the CEQ may particularly disadvantage some Specialist International Medical Graduates (SIMGs). Although there was a lack of consensus among stakeholders on possible alternate formats for the CEQ, most stakeholders who contributed to the RANZCP internal consultations did not believe that the CEQ should be retained unchanged in its current format. There was also strong support for reviewing the skills intended to be assessed by the CEQ and exploring alternate formats for assessing them.

The RANZCP Board has now requested that ACER conduct a focussed review of the current CEQ format to consider its effectiveness, fitness-for-purpose and ongoing role in the RANZCP assessment program.

1.2 PURPOSE AND OBJECTIVES OF THE REVIEW

The intended purpose of the CEQ, as specified by the College, is to assess written, analytical, integrative, critical and reflective thinking skills. It aims to achieve this by requiring candidates to respond to a quote that can be related to a fundamental (and typically complex) topic from the history and philosophy of psychiatry. The CEQ intends to draw on candidates' understanding of how different factors have influenced the trajectory of psychiatry as a discipline, their appreciation of the importance

of cultural contexts, and their understanding of ethical considerations. There is significant debate in the literature on the definition of critical thinking, how (and if) it should be taught, and specific measures that might provide an assessment of students' critical thinking skills (Moore, 2013; Tiruneh et al., 2014). Recognising that the CEQ may not be assessing these important skills effectively, RANZCP is interested in exploring other assessment formats. There may be alternative assessment formats that can offer a more holistic picture of the candidate's written, analytical, integrative, critical and reflective thinking skills as they apply to fundamental and complex issues in psychiatry, which may even be non-summative. This move aligns with shifts in contemporary medical specialist training and assessment, where varied assessment formats are utilised to provide longitudinal assessment data and accumulated evidence of learning, with more integration with work-based training and as part of a whole program of assessments (Schuwirth & van der Vleuten, 2011).

1.2.1 Project scope

In relation to the CEQ examination, ACER were asked to first evaluate the 2022 consultation findings and feedback received by the RANZCP from the relevant committees, trainees and other relevant College Fellow stakeholders. A desktop review of the existing material, and a concurrent review of the RANZCP curriculum learning outcomes and learning activities focused on the development of these skills would inform an articulation of the content and skills currently being assessed through the CEQ format. The project was also to be informed by contemporary directions for assessing such skills in comparable psychiatry specialist medical training contexts. After completing this preliminary work, RANZCP requested that ACER conduct its own consultations with stakeholder representatives, to seek further independent information and evidence around the CEQ.

Although the RANZCP recognised the possibility of ACER recommending that the CEQ be retained, RANZCP requested that ACER also identify other relevant alternative assessment formats that could assess the CEQ construct. If fit-for-purpose alternatives were possible, concrete examples of the assessment format were requested. An overarching objective of the review was also to give consideration to prevailing concerns around the 'burden of assessments'. This aligns with contemporary directions in health professions assessment, to transfer away from a single high-stakes assessment to a more integrated, holistic, longitudinal program of assessments.

In relation to the MEQ examination, the pass rates continue to be low despite decoupling of the previous Essay-style examination (where the MEQ and the CEQ were delivered in conjunction). As part of the current review, RANZCP also sought guidance from ACER in exploring approaches to improving the quality of the MEQ examination. As this component of the project was quite separate from the CEQ review, the MEQ review is presented as a comprehensive Appendix.

1.2.2 Deliverables

In terms of key deliverables for the whole project, ACER were asked to:

1. Clearly articulate the construct and critical thinking skills currently being assessed by the CEQ examination.
2. Review the alignment of the curriculum learning outcomes for the RANZCP training program that relate to these skills.
3. Review the mode of delivery and the training stage(s) where these skills are learnt.
4. Evaluate the outcomes from the RANZCP internal consultations with the stakeholder representatives.
5. Recommend an acceptable alternate assessment format(s) to assess the skills that the CEQ aims to assess and which meets the assessment requirements of feasibility, validity, reliability, and fitness for purpose.
6. Recommend the criteria/rubric broad criteria for meeting the standard of the assessment.

7. Provide a couple of samples and examples of the assessment format suggested.
8. Provide advice on possible ways to improve MEQ pass rates and possible online delivery format.

Deliverables 1 – 5 are presented in the body of the report. Deliverables 6 and 7 are presented in Appendix A. Deliverable 8 is presented in Appendix B.

2. METHODOLOGY

2.1 METHODOLOGY FOR THE PROJECT

The project was split into two main Phases, as per the key activities and timelines given in the Project Plan.

Phase 1 – Initiation, desktop and preliminary evaluation.

The ACER project team met with RANZCP to confirm project timelines and the project approach. RANZCP transferred background papers relating to the CEQ review and outcomes of the RANZCP consultation process through ACER's secure file transfer platform. The ACER project team reviewed these documents in line with the project scope, conducted the preliminary evaluation of the material, and delivered an Interim Report.

Phase 2 – Consultation and Final Report development.

The ACER project team then began a thorough consultation process, which involved one-on-one consultations with a range of stakeholders identified by the College. Eighteen individual consultations were conducted through June and early July 2023. Stakeholders included a range of members from the Written Examinations Subcommittee, the Committee for Training, the Committee for Examinations, Directors of Training, SIMG representatives, and trainee representatives. These consultations aimed to identify stakeholders' perspectives on the perceived key strengths and weaknesses of the CEQ format, and to gather insights about potential new formats for the CEQ. ACER then began developing this Final Report, including recommendations for options for the format of the CEQ and methods for improving the quality of the MEQ. Once possible assessment formats were identified, ACER staff also developed some sample assessment items and broad rubric criteria.

2.2 EVALUATIVE FRAMEWORK

The findings of the assessment reviews undertaken by ACER are underpinned by several key concepts and principles drawn from the contemporary literature on assessment best practice, as well as contemporary theories of assessment practice relevant to medical education and speciality training contexts. The definitions of each concept and the approaches taken in applying them are described in the sections below. These principles underpin the review conducted by ACER in relation to the CEQ and MEQ.

2.2.1 Assessment and learning

This principle aims to ensure that assessment and learning are integrated. All assessment should provide rich, meaningful and timely feedback to trainees and provide feedback on performance that contributes to trainees' learning (Sadler, 1998). Data can be used by supervisors/trainers to identify gaps in learning and to help them provide targeted teaching (Bennett, 2011). All assessments (both low and high-stakes ones) can and should provide useful and meaningful information for both trainees and supervisors. Detailed feedback on assessment performance allows trainees to improve their practice,

whereas pass/fail feedback provides very limited information for learning and development (Shute, 2008).

This notion of assessment and learning also aligns with a programmatic approach to assessment, which treats assessment as a whole program, rather than as isolated or ad hoc assessment instruments (Schuwirth & van der Vleuten, 2011). The strength of an assessment program is thus judged by the program as a whole rather than by the properties of single instruments. A variety of well-chosen tools can be used to provide adequate sampling across the breadth and depth of the curriculum.

2.2.2 Valid, aligned and fit-for-purpose assessment

The program outcomes, curriculum content, teaching and learning strategies, and assessment and examination processes all need to be aligned (Biggs, 2003). This alignment assures that content in the curriculum is linked to the stated learning outcomes of the program. If learning is a key focus of an assessment program, it is even more crucial that the assessments align to desired program outcomes. This is particularly important as it is a well-accepted adage that assessment drives learning. The process of blueprinting is a key tool to achieve constructive alignment (Cumyn & Harris, 2012).

An assessment is fit-for-purpose when the methods used to assess a particular learning outcome are relevant, appropriate and focused on measuring the achievement of that outcome (van der Vleuten et al., 2012). There are many aspects to assessment quality. Prime among these is the key notion of assessment validity. Historically, validity has been thought of as intrinsic to the assessment, so that educators commonly spoke of one assessment tool (e.g. MCQ) being more or less valid than another (e.g. OSCE). Today, validity is more commonly considered in terms of how assessments are used and the broader evidence which supports (or not) the conclusions and decisions based on those assessments (Cook et al., 2015; St-Onge & Young, 2015).

Another facet to assessment quality is the criterion of psychometric robustness, defined as the quality of the psychometric properties of an assessment. The reliability of an examination is the degree to which an assessment instrument produces stable and consistent results. Examination scores can never be completely reliable; however, in a high-quality assessment, the contribution of measurement error to candidate scores should be low. In a program of assessment, there will be a variety of different assessment methods deployed, each designed to consider issues such as validity, reliability, feasibility and acceptability. Essentially, consideration should be given to whether each selected assessment type adequately assesses the intended relevant content.

2.2.3 Collaborative construction of standards

Standards can be defined as the 'degree of academic achievement established by authority, custom, or consensus and used as a fixed reference point for reporting a student's level of attainment' (Sadler, 2013, p. 15). Determining performance standards should be a constructive and collaborative process. Having well-articulated performance standards helps improve the transparency of assessment and student learning (Hendry et al., 2012).

As assessment standards are socially constructed, assessors need to engage in dialogue about standards as part of a broader understanding of the process of assessment (HEA, 2016). Further, this cannot be a 'one-off' event, as the expectations for appropriate standards evolve over time. The process of standard setting, employed in medical health professions, is a robust approach to setting performance standards.

2.2.4 Defensibility of professional judgements

Professional judgement by content experts is an intrinsic element of assessment. Confidence in such judgements relies on developing appropriate standards for performance and regularly reviewing these standards. Professional judgements should be continuously reviewed through collaborative, critical reflection to ensure that they are reliable, consistent, transparent, fair, and ultimately defensible (Bloxham et al., 2016). Moderation techniques and forums for deliberating on performance have been shown to improve the assessment literacy of assessors and their ability to apply standards consistently and correctly (Bloxham, Hughes et al., 2016). Other assessment processes, such as double marking and examiner calibration, can reduce examiner variability and minimise potential bias.

2.2.5 Fairness and consistency

A program of assessment must be defensible (in terms of the assessments being valid, reliable, robust, etc.) and transparent (which means that fair and documented processes are used in making decisions about candidates' ability to progress). Fairness relates to the degree to which the assessment provides an equivalent measure of candidate ability independent of the personal characteristics of the candidate that are not relevant to the assessment. At the item level, this includes issues around potential bias in assessment. At the process level, fairness relates to the consistency of candidate experience and evaluation in the assessment process.

2.2.6 Limits and feasibility of assessments

Assessment can only do so much, and complete precision can never be fully attained. As such, there is no one ideal assessment method for all circumstances. In essence, the process of assessment should aim at optimisation (van der Vleuten, 1996). It is also important that any changes to assessments and assessment processes are feasible within a particular context. Assessment processes need to embrace this heterogeneity and find ways of incorporating best practice in unique contexts.

3. FINDINGS FROM DESKTOP CEQ REVIEW

3.1 EVALUATING THE RANZCP STAKEHOLDER CONSULTATIONS

As part of the College's approach to evaluating the role of the CEQ in the RANZCP program of assessments, the College initiated several internal rounds of consultation with stakeholder groups. The focus of discussion with stakeholder groups was an internal document critically evaluating the CEQ and proposing several options for assessing critical thinking skills in different formats. The discussion paper was initially considered by the Examinations Review Steering Group (ERSG) who recommended review and feedback from the Directors of Training (DOTs), the Committee for Training (CfT), the Committee for Examinations (CfE), the Committee for Specialist International Medical Graduate Education (CSIMGE) and the Trainee Representative Committee (TRC). Aggregated feedback from these committees in conjunction with written feedback was designed to form the basis of a submission to the Education Committee (EC) to consider potential changes to the CEQ. As part of the interim report, we undertook an initial evaluation of these consultations. Major themes arising from these consultations are briefly summarised below. Our analysis of the themes in the internal consultations was subsequently used to guide our independent consultations with RANZCP stakeholders.

Each of the stakeholder groups considered each of the options outlining potential formats for assessing critical thinking skills (Table **Error! Reference source not found.**). These options comprised: 1) no change to the CEQ, 2) short constructed response tasks, 3) a combination of home-based and exam-based assessment tasks, 4) home-based assignments and 5) workplace-based assessment (WBA). A summary of the trends in stakeholder perspectives across the five options is shown in the table below (where ✓ represents general endorsement for the option, ✕ represents a lack of endorsement and ✕✓ represents both positive and negative views of the option. There was no information on the perspectives of the CSIMGE for Option 4 (denoted as a '?')).

Table 2: Suggested alternate CEQ formats and the views of stakeholders in the internal consultations

	Option 1 <i>Retain CEQ</i>	Option 2 <i>Short Constructed response</i>	Option 3 <i>Home and exam-based assessment</i>	Option 4 <i>Home-based assignments</i>	Option 5 <i>WBAs</i>
CfE	✓	✕	✕✓	✕	✕
DoTs/CfT	✕	✕✓	✕✓	✕	✕
CSIMGE	✕	✓	✓	?	✕
TRC	✕	✕	✕	✓	✓

Overall, it is clear there is minimal support for retaining the CEQ in its current format, with only the CfE overwhelmingly supporting this option. However, there is no consensus among stakeholders for any of the suggested alternate formats discussed. For instance, while the CSIMGE were highly supportive of a short constructed response format, the TRC believed that this approach would be burdensome and instead favoured workplace-based assessments. The support for the CEQ in its current format expressed by the CfE emphasised the unique and fundamental skills the CEQ was believed to capture that are not assessed via other means as well as a lack of suitable alternatives to assess these skills. In contrast, the remaining stakeholders raised concerns such as questionable reliability and validity of

the CEQ, concerns about the authenticity of the CEQ format for assessing critical thinking, concerns about the fairness of the CEQ particularly for candidates for whom English is a second language, and suggestions that the CEQ may not be required (at all, or as part of high-stakes examinations). Overall, there was a strong desire to review how the CEQ fits into the overall program of assessment, to clearly define those skills the CEQ is designed to assess and how these are taught within the training program.

Different stakeholder groups clearly place more or less emphasis on the different criteria contributing to the utility of the CEQ and its proposed alternatives. Trainees particularly focused on acceptability of the current CEQ and possible alternate formats and considered assessment burden, impacts on trainee wellbeing and perceived fairness. Other stakeholders prioritised reliability and validity in arguing against the current CEQ format, although some also believed that the lack of standardisation was a barrier to assessing critical thinking through workplace-based assessment.

Notably, some additional themes emerge in consideration of alternative CEQ assessment formats. A theme of potential for assessment burden (both for trainees undertaking examinations and in terms of the resources required to develop, administer and assess examinations) emerged for some stakeholders. For instance, trainee stakeholders were concerned that short constructed response tasks to assess critical thinking might increase the burden on trainees and increase the burden of centrally administered examinations. Whereas trainee stakeholders' preference for workplace-based assessments of critical thinking were perceived as less burdensome.

A desire to limit the number of changes impacting trainees and to ensure sufficient time was allowed to clearly communicate impending changes to trainees, was also evident in the consultations. As part of this, some stakeholders noted the positive impact on pass rates of decoupling the CEQ from the MEQ and suggested that this needed to be monitored prior to planning any significant changes to the CEQ format.

The degree to which critical thinking skills are currently explicitly taught (and whether they should be taught) in the RANZCP training program was also discussed extensively by some committees. Some stakeholders did not believe that these skills were adequately represented in the training program curriculum or that critical thinking skills were explicitly taught. Others believed that explicit representation in the curriculum or specific training in critical thinking was not necessary. Instead they believed that critical thinking was fundamental to the work of the psychiatrist and did not need to be explicitly stated.

Overall, despite significant variability in stakeholder views of the original CEQ and potential alternate assessment formats, the need to carefully review and define the critical thinking construct was widely endorsed.

3.2 THE UTILITY OF THE CEQ

Although the views of RANZCP stakeholder groups surrounding the purpose and format of the current CEQ varies, the most recent Examination Report is clear about its intended purpose.

The purpose of the CEQ assessment is to test skills considered essential for consultant psychiatrists. The skills align with CanMEDs roles, and allow candidates to demonstrate the ability to evaluate and critically appraise a proposition relevant to psychiatry, apply evidence-based assessment, and demonstrate a capacity for balanced reasoning.

As usual, the candidates were provided with a prompt in the form of a quote. They were expected to address the quote, critically discuss it in essay form (including a consideration of contrasting viewpoints), and provide a conclusion.¹

Included in the scope of this project is the task of providing a clear re-articulation of the content and skills being assessed through the current CEQ format, especially in the context of contemporary best-practice in assessment. This is a question of whether the CEQ remains fit-for-purpose, which has been identified as a functional definition of quality (Schuwirth & van der Vleuten, 2011). Before undertaking this re-articulation of the assessment construct, the overall utility of the CEQ is examined.

Assessment is an optimisation problem. No one assessment method is perfect in every aspect, every assessment method has its strengths and weaknesses, and the utility of any assessment approach depends on the purpose of the assessment process. A utility formula outlined by van der Vleuten (1996) is a helpful framework for reviewing specific assessment instruments. The five criteria identified as contributing to the utility of an assessment are validity, reliability, educational impact, acceptability and cost (also interpreted as feasibility). The main argument given by van der Vleuten is that no one assessment will be perfect on all quality criteria. Assessment optimisation is naturally a trade-off, and decisions on which criteria to maximise will depend on and be influenced by contextual factors.

Summative high-stakes examinations in speciality training (such as the CEQ) typically maximise validity and reliability, as high-stakes decisions around candidate progression (including awarding fellowship or, conversely, removal from a training program) must be based on robust information to be fair, consistent and defensible. Lower-stakes assessments, such as work-place based assessments typically accept lower levels of reliability (with greater variability), in the interests of maximising the learning function of the tool and its educational impact.

Below, as part of our desktop review, we have evaluated the CEQ against the five criteria of validity, reliability, educational impact, acceptability and cost/feasibility.

3.2.1 Validity

Validity is a fundamental concept in assessment. Colloquially, validity is typically distilled into 'fitness-for-purpose'. An assessment is deemed fit-for-purpose when the methods used to assess a particular learning outcome are relevant, appropriate and focused on measuring the achievement of that outcome (van der Vleuten et al., 2012). But validity as a concept has a longer history and can refer to many things and comes in many forms (construct, content, face, predictive, and so on). Today, the notion of validity is typically considered in terms of evidentiary frameworks. It refers to the amount of "theoretical and empirical evidence or confidence one has in the claims made based on data generated by an assessment process" (Tavares et al 2022). The validity of an assessment instrument is established through arguments based on substantive evidence.

For an assessment such as the CEQ, validity can be built through internal, external and statistical/psychometric methods.

Internal methods include consensus around the purpose of the assessment, drafting items with clear guidelines, and consistent blueprinting to ensure the items map to the agreed learning outcomes;

External methods include the use of peer review and iterative editing by the examination panel; and

¹ August 2022 CEQ Examination Post-examination Report

Statistical/psychometric methods include post-examination review using item analysis.

On each of these criteria, we would judge the CEQ as sub-optimal. That is to say, we see that improvements could be made in the internal, external and statistical/psychometric approaches to this specific examination.

Rather than outlining the precise areas for improvement here (indeed, some of these were noted in ACER's 2020 Examination Review Report), we instead want to focus on the inherent limitations of the CEQ format. The essay format currently rewards specialised writing skills. The exam becomes artificially more challenging for SIMGs and/or candidates where English is not their first language. These candidates may be able to demonstrate proficiency in the relevant skills in a different format, which undermines the potential validity of the current CEQ (and raises basic concerns of face validity). It is also unclear whether the writing skills assessed by the CEQ align with those required in psychiatric practice, which undermines the criterion validity of the exam. Even with developments in generative artificial intelligence, psychiatrists will still in practice require the ability to write logically and coherently. However, the relevance to psychiatric practice of being able to write a sophisticated and nuanced essay on a specific quote is doubtful.

3.2.2 Reliability

Another facet to assessment quality is the criterion of psychometric robustness, defined as the quality of the psychometric properties of an assessment. This is often interpreted as the reliability of an examination – the degree to which an assessment instrument produces stable and consistent results. Examination scores can never be completely reliable; however, in a high-quality assessment, the contribution of measurement error to candidate scores should be low. Metrics such as internal consistency and test-retest reliability are based on the generalisability of collected data. But reliability is too often interpreted as “an entity in its own right when it is nothing more than a statistical calculation for which interpretation should be heavily dependent on implementation, context and philosophical position” (Tavares et al. 2020). Instead, reliability can be thought of as a ‘sampling problem’. If you want a more reliable examination, make it longer; ask more questions.

This highlights a major flaw with the CEQ. It is an exam comprised of one question, represented as one quote. If a candidate is ‘thrown’ by the quote (if they don’t quite appreciate its meaning, its potential dimensions, its context) then they won’t demonstrate proficiency in the assessed skills (even if they do possess them). No classical item statistics were sighted as part of the CEQ review (the only available data was related to CEQ marking and standard setting). To improve this reliability concern, any future assessment that targets the CEQ construct should involve multiple questions/cases/items.

3.2.3 Educational Impact

A third criteria in van der Vleuten’s utility formula is the educational impact of an assessment. It is a well-known adage that assessment drives learning. Running an exam like the CEQ influences the behaviour and learning of trainees and SIMGs. If the CEQ aligns well to practice and the desired learning outcomes of the training program, then there is no real issue. But if there is poor alignment to training (discussed later in this report) or if there are other unintended educational impacts, then there are grounds for concern on this criterion.

It is probable that candidates with a strong command of written English and well-developed essay writing skills could do well in the CEQ with little preparation, and thus won’t necessarily learn much from the examination preparation process. They may read more widely than they would have on issues

relating to psychiatric practice, and discuss these issues in learning activities such as journal clubs. Conversely, candidates who believe they will struggle with the CEQ may learn to write better essays in preparing for the CEQ. However, this does not necessarily mean that they will improve their critical thinking skills. Instead, they are likely to learn techniques needed to pass the CEQ. We sighted some educational support material for candidates, along with several examples of strategies or ‘tips and tricks’ for the CEQ. However, learning test techniques will not necessarily improve the desired acquisition of the relevant construct. Some of these resources were readily available on the RANZCP website, while others were easily found with search engines. There are also non-College endorsed educational courses available (at a high price) that promise to help candidates improve their essay writing skills to pass the CEQ.

3.2.4 Acceptability

Acceptability is an important criterion for an examination. If the examination is not acceptable, then its utility is reduced, as the legitimacy of inferences from the assessment data is called into question. We see acceptability as, in part, a corollary of validity, reliability and educational impact. If these first three criteria meet the required purpose of the assessment, it is likely to be acceptable to all constituents. There are, of course, certain exceptions to that. For instance, if the CEQ required 10 essays on different quotes it would certainly increase the reliability of the instrument, while the acceptability would undoubtedly be reduced.

As detailed earlier in this report, the CEQ is a divisive examination. Feedback collected from a wide-ranging consultation has determined that stakeholders’ views regarding the value of the CEQ and the format in which critical thinking skills might be assessed varied significantly. Although many stakeholders believe the skills being assessed are relevant to psychiatry, stakeholders raised issues with a perceived lack of validity, questionable reliability, and concerns around potential disadvantage towards SIMGs. The clear lack of acceptability, exemplified by the significant variability in stakeholder views, calls for a careful review of the current CEQ format.

3.2.5 Cost/Feasibility

The cost of an examination does not neatly align with the first four criteria, but it represents an important pragmatic consideration. If an assessment instrument costs too much to implement, even with excellent validity, reliability, educational impact, and acceptability, then it won’t be an optimal assessment solution. We think ‘feasibility’ is a more general criterion than cost, and one that is crucial to thinking about assessment solutions in context.

The current delivery of the CEQ appears to be feasible to continue. Its exact cost (both in terms of money and time) is unknown to us, so we cannot comment. However, despite the current CEQ being, on the face of it, feasible, the continuation of the CEQ in its current form appears, increasingly, infeasible.

3.3 ARTICULATING THE CEQ CONSTRUCT

3.3.1 Reviewing the construct via the blueprinting

To clearly articulate the CEQ construct, we began by reviewing the blueprinting (mapping) between the Fellowship Competencies and the CEQ marking guide.

The Fellowship Competencies appear to be closely aligned to the CanMEDS Framework. They are described in quite generic terms, aside from explicit connections to the practice of psychiatry in the level descriptors. The competencies assessed in any one administration of the CEQ are not completely consistent. There is some variation across sittings in which Fellowship Competencies are allocated to the paper, presumably based on the quote chosen (Table 3).

Table 3: *Fellowship Competencies assessed in different CEQ Sittings*

CEQ Sitting	Competencies being assessed
Feb, 2022	1, 3, 5, 6, 7 and 9
Aug, 2022	1, 3, 5, 6, 9 and 10
Aug, 2021	1, 3, 5, 8, 9 and 10

When a competency is being assessed by the CEQ, not all aspects of that competency are necessarily assessed. For example, in the February 2022 CEQ sitting, candidates were asked to critically discuss the following statement from different points of view and provide their conclusion, using essay form:

The therapeutic relationship is a microcosm of broader power relations, and hence suggestions of an interdependency between clinician and patient become unsettling.

Their essay was graded against Fellowship Competency 1, 3, 5, 6, 7 and 9. We note that the alignment between proficiency level descriptions and Fellowship Competencies varies throughout the marking guide. For example, for Fellowship Competency 1, there was strong alignment between the proficiency level descriptions and the competency to be assessed (see Figure 1).

Fellowship Competency 1. Communicator – Weighting 10%	
The candidate demonstrates the ability to communicate clearly. Spelling, grammar and vocabulary are adequate to the task, and able to convey ideas clearly.	Proficiency Level
The spelling, grammar or vocabulary significantly impedes communication.	①
The spelling, grammar and vocabulary are acceptable, but the candidate demonstrates below average capacity for clear written expression.	②
The spelling, grammar and vocabulary are acceptable, and the candidate demonstrates good capacity for written expression.	③
The candidate displays a highly sophisticated level of written expression.	④
	⑤

Figure 1: *Fellowship Competency 1. February 2022 CEQ.*

However, in the case of Fellowship Competency 3, there was only partial alignment between the proficiency level descriptions and the competency to be assessed. The reference to research literature was not explicitly present in the proficiency level descriptors, but may be implied through the use of the term 'relevant knowledge' in the descriptors for Levels 1 and 2. The ability to consider counter

arguments or argue against a proposition, was not mentioned explicitly in any of the proficiency level descriptors (see Figure 2).

Fellowship Competency 3. Medical Expert, Communicator, Scholar – Weighting 20%	
<i>The candidate is able to identify and develop a number of lines of argument that are relevant to the proposition. The candidate makes reference to the research literature where this usefully informs their arguments. Includes the ability to consider counter arguments and/or argue against the proposition.</i>	Proficiency Level
There is no evidence of logical argument or critical reasoning: points are random or unconnected or simply listed.	①
There is only a weak attempt at supporting the assertions made by correct and relevant knowledge OR there is only one argument OR the arguments are not well linked.	②
The points in this essay follow logically to demonstrate the argument and are adequately developed.	③
The candidate demonstrates a sophisticated level of reasoning and logical argument, and most or all the arguments are relevant.	④
	⑤

Figure 2: Fellowship Competency 3. February 2022 CEQ.

Overall, the articulation of the different Fellowship Competencies are rather generic in nature. This makes sense, as they align to the generic CanMEDS non-medical expert domains.

The College should be commended for attempting to assess these non-medical expert domains in a summative fashion. This is rare in specialty training, with non-medical expert competencies usually assessed formatively, or rather implicitly in summative assessments. However, the mapping of the RANZCP Fellowship Competencies to the CEQ feels somewhat forced, which is demonstrated by the lack of alignment in the proficiency level descriptions. It is plausible that people with critical thinking and essay writing skills could perform well in the CEQ even if they are not in the RANZCP training program (or indeed, even if they are not medically trained). This highlights that the current CEQ has low validity, and is thus not fit-for-purpose.

The levels-based marking guide used to assess the CEQ is good practice for identifying a hierarchy of performance-quality in the different assessment domains. Level 3 seems to correspond to the minimum level of adequate performance, or the performance expected of a minimally competent candidate. This observation correlates with the standard setting datasets sighted, as the standard setting consensus routinely lands around level 3.

From this process, we have seen that the CEQ is broadly intended to assess:

- the ability to evaluate and critically appraise a proposition concerning psychiatry,
- the ability to communicate clearly in extended written form,
- the capacity for balanced reasoning and considering different points of view,
- the ability to identify and develop lines of relevant argument based on evidence,
- the ability to draw a conclusion that is justified from their arguments,
- the ability to demonstrate an understanding of models of health and illness,
- the ability to demonstrate an understanding of cultural sensitivity and the cultural context of psychiatry,
- the ability to demonstrate an appropriate awareness of broader social, cultural, philosophical and relevant ethical issues,
- the ability to demonstrate understanding of patient-centred care, and
- the ability to express views in written form under time pressure.

3.3.2 Articulating the current CEQ construct

In attempting to articulate the construct and critical thinking skills being assessed by the CEQ, we propose articulating two constructs – the first defines what the CEQ is currently assessing, and the second defines what the CEQ should (or could) be assessing. With this first articulation, we anticipate

that many will disagree that this is what the CEQ is or should be assessing. However, we conclude that this is the construct currently being assessed by the CEQ in its current form.

Construct articulation 1: What is *currently* being assessed by the CEQ?

The construct being assessed is the candidate's ability to write a sophisticated and nuanced essay on a quote relating to psychiatry, by contrasting different points of view and presenting a conclusion. This encompasses written communication, argumentation, the ability to draw on relevant examples and evidence, and developing coherent ideas that flow logically.

Proficiency is demonstrated by connecting the quote to diverse perspectives relating to psychiatric healthcare and practice, including (but not limited to) historical, social, cultural, philosophical, and ethical dimensions.

3.3.3 Re-articulating the construct as it should be?

The first articulation will not necessarily align with the RANZCP Board's expectations. The skills that the CEQ is intended to assess seem to go further than written communication and essay writing. The construct should perhaps be focussed more on the ability to *think* critically in a sophisticated and nuanced manner in relation to psychiatry. The first construct does not really capture the required "skills considered essential for consultant psychiatrists". Independent of the CEQ assessment, it is unlikely that psychiatrists will have to evaluate and critically appraise a proposition relevant to psychiatry in an essay format. Further, candidates may actually possess critical thinking skills but may be hindered from conveying them due to the written format.

We propose that the construct should place less emphasis on essay writing *per se*. Essay writing is a specialised skill that has to be learned and practised. Some trainees will possess such skills on entering the program. Others may have had little exposure to essay writing in recent years in any educational capacity. If the specific skill of essay writing is a skill deemed important for trainees' professional practice, essay writing could be considered as part of a new selection assessment process or as part of a take home assignment. However, in the latter, any take home assignment needs to take into account the proliferation of AI tools and candidates using these tools to develop their submissions.

A rearticulated CEQ construct should centre more around thinking. It is likely that proficient candidates would readily recognise multiple perspectives on an issue, appreciate drivers of perspectives, recognise potential tensions and examine contentious aspects of an issue. And of course, a proficient candidate would likely understand the issues in a range of contexts. But the expression of the construct should not be limited to written communication, and to conveying ideas in a once-off essay format.

Thus, we propose the following revised articulation of the construct.

Construct articulation 2: What *should* be assessed?

The construct to be assessed is the candidate's critical thinking ability in the context of psychiatry, encompassing effective communication, argumentation, the ability to draw on relevant examples and evidence, and develop coherent ideas that flow logically.

Proficiency is demonstrated through a nuanced understanding of diverse perspectives relating to psychiatric healthcare and practice, including (but not limited to) historical, social, cultural, philosophical, and ethical dimensions. This involves the capacity to recognise the role that psychiatry plays in a range of contexts, analyse tensions, explore potentially competing views, and substantiate ideas with evidence, exhibiting a sophisticated and comprehensive approach to the subject matter.

In this version, we deliberately avoided using the words 'write' or 'written' in describing the skills required and the construct is not limited to responding to a single quote. Through evaluating the consultations already conducted with stakeholders, we anticipate that most would agree that these skills *should* be assessed at some point in the training program. Psychiatrist educators would likely want to determine whether trainees and SIMGs can do this at some point in training. If this is the case, then the current CEQ is certainly not fit-for-purpose and alternative assessment formats should be explored.

3.4 ALIGNMENT TO THE RANZCP CURRICULUM

In a high-quality assessment program, the program learning outcomes, curriculum content, teaching and learning strategies, and assessment and examination processes all need to be aligned (Biggs, 2003). This alignment assures that content in the curriculum is linked to the stated learning outcomes of the program. The process of blueprinting is a key tool to achieve constructive alignment in assessment (Cumyn & Harris, 2012).

The RANZCP training program contains a detailed Syllabus for stages, Fellowship Competencies document, and Learning Outcomes mapped to Stage. The CEQ is used as a high-stakes assessment tool in Stage 2 of the training program. However, the website notes that candidates are eligible to apply for the CEQ once they have completed 18 months of FTE training. The website also notes that "The RANZCP recommends that you take the CEQ exam in Stage 3, as the exam is set at the standard expected at the end of Stage 3."²

Stage 2 of the training program is designed to enable the trainees to apply their knowledge and skills within a variety of settings and with diverse patient populations. The syllabus for Stage 2 of the training program explicitly discusses psychiatric healthcare and practice, including the topics of *Ethics*, *Cultural competence*, *History* and *Indigenous Australians/Māori mental health*.

² <https://www.ranzcp.org/training-exams-and-assessments/exams-assessments/exams/ceq>, Accessed 30 May 2023.

However, it is not clear at this stage if the skills required to demonstrate critical thinking through a sophisticated and nuanced essay are being taught as part of the training program. Upon evaluation of the Learning Outcomes of Stage 2 of the program, trainees should have developed the ability to:

- develop management plans,
- develop and present professional development plans,
- present at journal club,
- perform professional presentations and case presentations,
- understand the principles of report writing,
- use verbal and non-verbal communication to suit a wide range of professional settings,
- communicate information in a variety of settings, and
- provide timely, structured and reasoned written reports and letters.

The ability to present written work in an essay style format is not explicitly in the learning outcomes for Stage 2. The 'Reflection' domain does mention "Engages in critical reflection and self-monitoring during clinical practice..." as a Stage 2 Learning Outcome, and "the ability to critically appraise and apply contemporary research, psychiatric knowledge and treatment guidelines to enhance outcomes..." as a Stage 3 Learning Outcome. Neither of these, though, are centred on the acquisition of critical thinking skills.

We thus see a gap between the skills currently being assessed in the CEQ and how these skills map/align to learning outcomes in the training program. This was echoed in the internal RANZCP stakeholder consultations mentioned earlier. Furthermore, we did not sight any structured learning activities or *ad hoc* learning opportunities for trainees or SIMGs that could represent learning moments for these skills. These observations align with trainee and SIMG perspectives that the critical thinking skills assessed by the CEQ are not adequately taught in the training program.

4. FINDINGS FROM ACER INTERVIEWS

4.1 RANZCP STAKEHOLDER INTERVIEWS

ACER undertook individual interviews with RANZCP stakeholders to provide comparative data for the consultations undertaken internally by RANZCP with several College committees. Our initial review of these internal consultations suggested significant variation among College stakeholders in the perceived importance of and commitment to the CEQ examination. We undertook consultations with 18 College stakeholders from June-July 2023. Those consulted reflected a broad range of perspectives including long-standing Fellows of the College, and recent Fellows and current RANZCP trainees who had completed the CEQ during their training. The stakeholders we consulted also represented a wide range of contexts from locations across Australia. These stakeholders had differing experiences with the CEQ from completing the examination as part of the training program, to developing, standard setting and marking the examination and to supporting trainees preparing for their examination as part of a supervisory role.

Overall, half of the stakeholders were supportive of the CEQ and strongly believed the skills assessed by the CEQ were fundamental. The remaining stakeholders were highly critical of the current CEQ and believed that the format must change (even if they believed in principle that the skills that the CEQ was intended to assess were important). Even among the stakeholders who supported the CEQ in its current format, most were amenable to exploring different formats if an acceptable alternate format could be identified. The following sections summarise the main themes arising from the interviews with stakeholders about the CEQ related to purpose, developing skills during training, trainee preparation and support, fairness and possible alternate formats.

4.1.1 Purpose of the CEQ

To elicit interviewees' perspectives on the CEQ, interviewees were asked to independently generate their conception of the purpose of the CEQ. They were subsequently asked to indicate whether they agreed with a RANZCP statement about the purpose of the CEQ:

The CEQ examines skills considered essential for consultant psychiatrists i.e. the ability to evaluate and critically appraise a proposition relevant to psychiatry, apply evidence-based assessment, and demonstrate a capacity for balanced reasoning.

Identified purposes of the CEQ included one or more of the following elements: to assess trainees' knowledge about broader issues in psychiatry, to assess trainees' capacity to communicate clearly in written English, to assess trainees' ability to critically analyse complex issues relevant to psychiatry, and to assess trainees' ability to communicate a reasoned argument about complex ideas with consideration to alternate perspectives.

However, the emphasis on these different elements and the extent to which the CEQ was believed to succeed as an assessment of these elements varied widely. Many stakeholders (including those who did not value the CEQ format) mentioned the importance of psychiatrists developing a sophisticated understanding of the broader social, cultural and historical influences on their discipline. Some regarded all aspects of the purposes identified above as fundamental to psychiatric practice and to becoming a psychiatrist. They believed that the CEQ assessed 'higher-order thinking' required as a psychiatrist and that the ability to communicate complex ideas clearly in written format was similarly a prerequisite for psychiatric practice. Nonetheless many stakeholders, while recognising merit in the intended purpose of the CEQ, were critical of the CEQ format. Some believed that the purpose of the CEQ was unclear,

or doubted whether its stated purpose was achieved in the current format. Others believed that its purpose was largely confined to assessing English language skills. The handwritten format of the CEQ and the structured essay style were regarded as impediments to reliably assessing aspects of candidates' knowledge, understanding or reasoning about complex issues in psychiatry. Writing an essay was not regarded as an essential competency for a practising psychiatrist and many stakeholders believed that the CEQ written format did not reflect the writing abilities required in their work.

For many stakeholders interviewed, the major impediment to meeting RANZCP's purpose statement for the CEQ was the perceived disconnect between the stated purpose of the CEQ and the examination format. A high-stakes examination that was a handwritten, timed, essay on a single proposition was regarded as not relevant to everyday practice by some stakeholders, even if they agreed that purposes such as high-quality communication and the ability to see an issue from multiple perspectives was important.

4.1.2 Fellowship competencies and the CEQ

The CEQ was believed to assess (or was intended to assess) competencies related to several of the CanMEDS roles. Medical Expert and Communicator were mentioned most often, but Scholar, Advocate, Professional, Collaborator and Leader were also occasionally mentioned.

For those stakeholders who identified challenges with the CEQ, more difficulties were identified in the link between the CEQ and Fellowship competencies. For some, the CEQ was regarded as limiting the assessment of written communication to a very specialised format that was less relevant to Fellowship competencies and not reflective of psychiatric practice. Others regarded the CEQ format as wholly unsuited to assessing Fellowship competencies.

4.1.3 Developing skills during training

Opportunities for trainees to access teaching and learning opportunities during the training program for the skills underlying the CEQ were regarded by all stakeholders as insufficient, variable or absent. Within this overarching perspective there were two clear sub themes; the perception that the skills underlying the CEQ did not relate well to trainees' day-to-day practice and thus required specific preparation to successfully pass the examination, compared with the view that the skills were fundamental to psychiatric practice and could be taught and learned as part of daily work.

Stakeholders identified a lack of formal teaching (such as classes on ethics, the history of psychiatry), the insufficiency of teaching resources provided by the College on the CEQ format, and variable learning opportunities in the workplace that often depended on individual supervisors. Many perceived the CEQ as disconnected from daily practice, with the written requirements of the CEQ not regarded as relating well to the writing required of psychiatrists. Trainees in particular did not believe there was a connection between what occurred in the workplace (including EPAs and other WBAs) and undertaking the CEQ. Others affirmed that teaching related to the CEQ was insufficient but argued that the CEQ was strongly related to everyday practice and that patient interaction and the requirements of the training program provided opportunities to provide more formal teaching related to the CEQ (even if it did not at present occur regularly).

4.1.4 Trainee preparation and support

The major theme identified in relation to providing preparation and support for trainees to successfully undertake the CEQ was variability. Preparation for trainees was described as variable, highly location and site specific, and dependent on the availability of consultants interested in education and the CEQ.

Some preparation specific to the CEQ was available in some locations, such as practice exams, lectures from consultants, CEQ tutorials, and regular essay writing with feedback. Such preparation was sometimes regarded as sufficient. In other locations, no specific CEQ preparation was provided. Some stakeholders referred to specific preparation activities (such as practice exams or debating activities) that used to occur but no longer did. The level of support provided to undertake the CEQ was regarded by the majority of stakeholders as not sufficient. The variable support for trainees depending on their locations was perceived as inherently unfair (particularly for trainees who might find the CEQ more challenging, such as SIMGs). In the absence of formal preparation and support trainees largely undertook self-directed learning and it is thought that many trainees pay for private CEQ preparation courses.

More central support from the College was regarded as important in addressing variable access to CEQ preparation and support for trainees across training locations. Although resources provided by the College (such as past CEQ papers) were regarded as helpful, they were also described as insufficient. Some stakeholders emphasised the need for formal coaching because they believed that the essay writing skills required for the CEQ were not formally taught in the program or as part of previous medical training or education. At the same time, some stakeholders suggested that delivering preparation specific to the CEQ suggested that the skills required of the examination were not part of day-to-day practice, but a specific exam skill learned simply to pass the examination.

4.1.5 Fairness

Stakeholders identified several aspects of the CEQ format that challenged the fairness of the examination because they were perceived to advantage or disadvantage specific groups. A concern about the lack of fairness of the CEQ for specific groups was expressed by the majority of stakeholders consulted. A further challenge to fairness (discussed in the previous section) related to variable access to preparation for trainees to undertake the examination.

Many stakeholders were concerned that the current CEQ format advantaged trainees whose first language was English and thus disproportionately disadvantaged specific groups, such as SIMGs. A further concern was that SIMGs could be disadvantaged because the cultural perspectives reflected in the CEQ were specific to the Australian and New Zealand context, which could reflect quite different perspectives to international contexts.

The essay format of the examination was also identified as a factor in discriminating against trainees whose first language was not English and against trainees who had not been explicitly taught these skills. Trainees who had exceptional essay writing skills and who had been taught how to structure essays and how to structure an argument were believed to have an advantage in the CEQ. Many stakeholders believed that the written expression required to successfully pass the CEQ was not a core element of psychiatric practice. Moreover, they asserted that English language ability was not what the CEQ was intended to assess.

The timed aspect of the CEQ was also identified as an impediment to fairness, particularly for trainees whose first language was not English, and an advantage for trainees who responded well to time pressure and who could manage their time systematically. The requirement to handwrite legibly under time pressure for the CEQ was also regarded as an impediment to fairness, potentially affecting some trainees (perhaps those who were older) more than others. For those who regarded handwriting the CEQ as unfair, converting the exam to a typewritten format was presented as a straightforward solution.

A contrasting perspective was that all psychiatrists practising in Australia needed to have a strong grasp of the English language in written and spoken form. Stakeholders with this view linked the written format of the CEQ to the expectations of written communication in psychiatric practice such as writing letters to General Practitioners following an interview with a patient. Others believed that the disadvantages experienced by SIMGs were not specifically related to English language capacity but reflected systemic issues such as the lack of supervision or support for SIMGs. For some stakeholders, the timed and handwritten aspect of the CEQ remained fair because the ability to write legibly and think critically under time pressure was regarded as core to psychiatric practice.

4.1.6 Alternate formats

Almost all of those consulted were open to exploring other formats for assessing skills currently assessed by the CEQ. However, as was the case in the internal stakeholder consultations conducted by the College, perspectives on possible alternate formats varied widely. A small number of stakeholders were satisfied with the CEQ in its current format and did not consider that a change of format was necessary or did not believe a change of format was necessary unless a compelling alternate format was proposed. Many considered that a range of alternate formats were possible, while also expressing a great deal of uncertainty about an acceptable approach. The main alternate formats proposed were workplace-based assessments and oral examinations; some believed that the skills assessed by the CEQ were already adequately assessed within the program and no replacement examination was necessary. Each of these suggested approaches had perceived strengths and weaknesses.

One group of stakeholders strongly believed that the skills assessed by the CEQ currently should be assessed in the workplace or were currently already assessed repeatedly (albeit, incidentally and implicitly) in the workplace. Nonetheless, there were concerns that increasing workplace-based assessments for supervisors would increase their burden (in the context of very limited time) and decrease the rigour of the assessment, as supervisors were not necessarily trained to complete these reliably and to provide high-quality feedback. Some stakeholders were also not in favour of workplace-based assessments to replace the CEQ because they were opposed to further reducing the summative examinations in the training program.

Another suggested alternate format was a summative oral examination. Several different potential approaches were proposed. These included a consultancy viva where candidates could be required to discuss multiple perspectives and examiners could probe their responses, or a multiple mini-interview style where candidates could rotate through a series of shorter topics. Some expressed concerns about losing the assessment of written skills should the CEQ be replaced with an oral examination. The ability to express ideas in writing in an essay format is highly valued by some stakeholders. These stakeholders believed that were the CEQ to be replaced by an oral examination that some form of written component should be retained (for instance submitting an essay with a subsequent oral examination to explore the trainee's thinking). However, others believed that assessment of writing skills occurred regularly in the workplace and other aspects of the assessment program (such as the Scholarly Project) also provided avenues to assess writing skills. Several interviewees were not in favour of a summative oral examination believing that such assessments would likely cause more anxiety for trainees and compound performance issues.

Very few stakeholders mentioned take-home assignments. Where these were raised it was in the context of being unable to ensure that the submission was the candidate's own work. A take-home

component followed by an oral exploration to ensure that it was the candidate's own work was suggested as a potential solution.

Finally, many stakeholders believed that the CEQ had the desirable effect of encouraging trainees to read widely and to engage with debated and challenging issues in their profession. They were reluctant to remove the CEQ as a summative examination because they were concerned that this significant benefit would be lost.

4.1.7 Summary

The consultations undertaken by ACER with RANZCP stakeholders reflect the variable perspectives on the CEQ identified during RANZCP's own internal consultations. Although some support for retaining the CEQ unchanged is evident, from the majority of stakeholders consulted there is strong support for better articulating the purpose of the CEQ, more strongly linking the purpose of the CEQ to the training program outcomes, providing better teaching and learning opportunities for trainees to develop the skills thought to underlie the CEQ, and considering alternate assessment formats to assess the relevant construct. With consideration to the complex relationships between stakeholders' views of the CEQ and the future development of this assessment, we sought a broader evidence base to inform decision making. Our first source of evidence was to identify whether similar assessments to the CEQ were evident in other relevant speciality training contexts. Thus, in the next section, we briefly review the curricula and assessments for training as a psychiatrist in the UK and Canada.

5. FINDINGS RELATING TO ALTERNATIVE FORMATS

5.1 INTERNATIONAL COMPARISONS

To identify assessment formats that might compare with the intention and format of the CEQ, we briefly reviewed the assessment programs of the Royal College of Psychiatrists in the UK and the Royal College of Physicians and Surgeons in Canada for the psychiatry specialty. We also briefly reviewed the curricula in both these contexts to identify learning outcomes that could map to the purposes of the CEQ identified in RANZCP statements and through stakeholder consultations.

5.1.1 Royal College of Psychiatrists UK

The Royal College of Psychiatrists in the UK implemented a new curriculum in 2022. For the purposes of this review, we focused on the newly implemented curriculum rather than the outgoing curriculum. Core training in psychiatry in the UK occurs over three years and follows two years of foundation training as a doctor following medical school. At the end of core training, psychiatric trainees undertake the Members of the Royal College of Psychiatrists Examinations (MRCPsych) that must be completed successfully to become eligible for a further three years of speciality training in psychiatry.

The core curriculum for psychiatric trainees in the UK includes high-level outcomes and key capabilities to be achieved under 16 key themes. Examination of the high-level outcomes and key themes suggests several elements that may overlap with themes identified in the consultations with key stakeholders against the intended purpose of the CEQ. For instance, RANZCP stakeholders sometimes alluded to the need for psychiatric trainees to grasp complex issues in psychiatry and to appreciate the multitude of factors influencing psychiatric practice. The core curriculum for the Royal College of Psychiatrists includes a theme of complexity and uncertainty that includes key capabilities that focus on analysing evidence and weighing up evidence to develop a balanced response. However, a further document outlining expected standards at each year of core psychiatry training clearly situates recognising complexity and uncertainty in the context of clinical work. For instance, a trainee at the end of the first year of training could be expected to recognise complexity in clinical cases and bring these to supervision, while a trainee at the end of the third year of training could be expected to manage complex cases and utilise their reflections on complex issues to institute management plans.

In the UK Royal College of Psychiatrists Core Training Curriculum, there is also an explicit learning outcome related to the history of psychiatry (“Demonstrate an understanding of the history of psychiatry, the development of diagnostic concepts and psychiatric treatments, as well as the profession, and the historical relationships between psychiatry and society”) against the high-level outcome related to: “Demonstrate skill in the psychiatric assessment, formulation, diagnosis and person-centred holistic management of an appropriate range of presentations in a variety of clinical and non-clinical settings”. Critical appraisal is mentioned in relation to analysis of research literature, but there is no reference to terms such as ‘critical thinking’.

As part of undertaking core training in psychiatry in the UK, trainees complete workplace-based assessments during training and a final examination at the end of the three-year program. For some of the workplace-based assessments available for review, each of the high-level outcomes for the psychiatry curriculum are listed in the marking criteria with ratings of progress against expected standards. In this way, the high-level outcome of understanding complexity and uncertainty is explicitly assessed. There is no specific reference to written communication skills in the curriculum (only

communication in general or verbal communication). Some of the workplace-based assessments do assess the quality of written expressions; however, this is in the context of making records on cases.

The MRCPsych examinations for core psychiatric training in the UK comprise two three-hour written papers that include a mix of multiple-choice questions and extended matching questions. There are no essay formats or other short answer written formats in these examinations. The first paper comprises assessment of the scientific and theoretical basis of psychiatry, while the second paper assesses critical review and the clinical topics in psychiatry. Critical review encompasses identifying research evidence and applying critical appraisal of such evidence to inform evidence based practice.

The final examination for the MRCPsych is the Clinical Assessment of Skills and Competencies (CASC). CASC is a 16-station clinical examination that is similar to an Objective Structured Clinical Examination (OSCE). The skills assessed in CASC are related to the high-level outcomes in the core curriculum. All of the curricula for speciality training in psychiatry retain the theme of uncertainty and complexity. Often the key capabilities expected within this theme relate to the ability to provide psychiatric care in situations of complexity. On other occasions, some aspects of the curriculum reflect the requirement that trainees demonstrate an understanding of complex issues that might impact psychiatric practice. For instance, both the core psychiatry and forensic psychiatry higher speciality curriculum have a key capability: “Demonstrate an understanding of individual variation and the impact of social, cultural spiritual and religious factors, including effects of deprivation, discrimination and racism.” This key capability resonates more with the breadth and depth of understanding captured in some perspectives of the purpose of the CEQ. However, there are explicit links to applying these understandings in the context of psychiatric practice, rather than articulating such understanding in a standalone format decontextualised from the patient experience.

5.1.2 Royal College of Physicians and Surgeons Canada

Psychiatry residents in Canada undertake a competency-based curriculum and complete examinations to become certified by the Royal College of Physicians and Surgeons Canada (RCPSC). Several aspects of the psychiatric competencies resemble some aspects of the suggested purposes of the CEQ. As in the Royal College of Psychiatrists in the UK, complexity and uncertainty in psychiatric practice is embedded in a learning outcome within the medical expert role: “Recognize and respond to the complexity, uncertainty, and ambiguity inherent in the practice of Psychiatry”. The history of psychiatry is also a required competency against medical expert. There are also several learning outcomes distributed across a range of CanMEDS roles that arguably would constitute knowledge on which candidates would draw for the CEQ. For instance, the Health Advocate role includes the learning outcomes: “Describe the ethical and professional issues inherent in health advocacy, including altruism, social justice, autonomy, integrity, and idealism”, and “Demonstrate an appreciation of the possibility of conflict inherent in their role as a health advocate for a patient or community with that of manager or gatekeeper”, the Medical Expert role includes the learning outcome: “Recognize and respond to the ethical dimensions in Psychiatric decision-making” and the Professional role includes the learning outcome: “Recognize and appropriately respond to ethical issues encountered in Psychiatry”. There are also several learning outcomes related to communicating effectively in written form, but this relates wholly to conveying information about a psychiatric encounter. Again, critical appraisal and critical evaluation are mentioned in relation to analysis of research literature, but there is no specific reference to terms such as ‘critical thinking’.

Candidates for the RCPSC complete a final examination comprising a written and a clinical component. The written component consists of 130-150 Multiple Choice questions and the clinical component

comprises a six station Objective Structured Clinical Examination each of 20 minutes duration. The MCQ examination is weighted towards the Medical Expert CanMEDS role; however, the classification of examination content suggests that 0-5% of the marks could be allocated to critical analysis and to special topics (stigma, culture). The content of the clinical examination suggests that it could cover content that includes writing consultation letters or reports, evaluation of scientific literature and critical appraisal. The critical analysis and appraisal competencies mentioned in regards to these examinations appear to be largely related to critical analysis of the scientific literature and clinical evidence. Three competencies related to critical appraisal of evidence and their application to clinical practice are included as part of the CanMEDS competencies under the Scholar role.

Psychiatry training experiences for the RCPSC include formal instruction in the history of psychiatry and other training experiences in the principles of critical appraisal and literature review and in aspects of ethics. Psychiatry training in Canada utilises Entrustable Professional Activities (EPAs) as part of a Competency by Design framework. The available EPAs for Psychiatry suggest that review of written information in relation to a clinical case comprises one EPA, as does critical appraisal and literature search. No other assessments or learning outcomes that transferred well to the intention of the CEQ were identified in this psychiatric training setting.

5.1.3 Summary

A brief examination of the curricula and examinations for specialty training in psychiatry in the UK and Canada did not identify any standalone written assessment format analogous to the CEQ. Aspects of both these curricula resonate with themes of complexity and uncertainty, understanding of the broader historical context of psychiatry and of the multitude of factors that might influence psychiatric care. These learning outcomes are also well-connected to understanding their influence on the provision of care to patients and to integrating broader understanding to improving outcomes for patients. As far as can be determined, there is no clear articulation of the development of critical thinking skills as a core component of the development of psychiatrists in these curricula. Moreover, the level of understanding required of trainees of complex issues related to psychiatry are often at the lower level of 'understand', rather than higher levels such as 'Evaluate', 'Analyse' or 'Critique' (except perhaps for the RCPSC reference to critical analysis and evaluation). In these contexts, deeper exploration of trainees' understanding of the history of psychiatry and complex issues affecting patient care and outcomes appear to be assessed through a combination of workplace-based assessment and oral assessment related to patient scenarios (rather than in a stand-alone format or disconnected from clinical cases).

5.2 CONSIDERATIONS FROM PROGRAMMATIC ASSESSMENT

The scope for the current project drew attention to prevailing concerns in relation to the 'burden of assessments' and flagged the potential transition by the RANZCP from single high-stakes assessments to more integrated, holistic and longitudinal programs of assessments. This suggestion aligns with contemporary assessment trends and directions.

5.2.1 Programmatic Assessment

The medical education literature has seen the steady rise over the past fifteen years of an approach to assessment known as 'programmatic assessment'. A programmatic approach to assessment treats assessment as a whole program, rather than as isolated or ad hoc assessment instruments (Schuwirth & van der Vleuten, 2011; Prideaux, 2016). In programmatic assessment, the design and utility of the

assessment program as a whole is emphasised, rather than focusing on the adequacy of individual assessments of performance (van der Vleuten & Schuwirth, 2005). This is because a program of assessment recognises that assessing complex competencies requires a range of measures and cannot be adequately learned and assessed through single assessments (van der Vleuten, Heeneman & Schuwirth, 2017).

Conceptualising assessment in this way means that a range of assessments purposefully selected may comprise a program, including those usually considered less standardised or less reliable, because these assessments fulfil a clear purpose in the overall program. Each individual assessment datapoint contributes to the evidence base for determining competence. Progression decisions are not made solely on the basis of one assessment instrument (such as an exam). Instead, accumulated evidence is reviewed by a committee of experts for decision-making purposes when there is enough evidence on the learner to inform robust decisions (van der Vleuten et al., 2015).

Although programmatic assessment approaches have become highly regarded in medical education, the philosophy of such approaches contrasts significantly with traditional summative, mastery-based approaches to assessment and learning. The substantial shift in orientation required to embed a programmatic assessment approach means that implementation is often challenging (van der Vleuten et al., 2015; Pearce & Prideaux, 2019). For example, the traditional formative/summative dichotomy is replaced with a continuum of stakes, from low- to high-stakes. This requires a shift in thinking for those who may be accustomed to a traditional assessment approach. Each individual assessment datapoint contributes to the evidence base for determining competence. Accumulated evidence is reviewed by expert judges for decision-making purposes.

Programmatic assessment thus removes pass/fail decisions from single assessment moments. Instead, rich assessment information is gathered on candidates using a wide variety of tools. The data in combination should provide a longitudinal profile on the learner's development (Heeneman et al., 2015). From a decision-making perspective, gathering rich assessment information across formats provides a clearer picture of candidate performance and enhances the 'trustworthiness and defensibility' of decisions. In a programmatic approach, high-stakes decisions (such as progression between stages) should be based on review of large volumes of evidence of performance rather than on individual assessments.

5.2.2 A philosophical shift away from individual instruments (such as the CEQ)

Focussing on the properties of individual instruments has been the mainstay of assessment culture in medical education for many years – dubbed a 'psychometric' approach in which the quality of an assessment system is judged by the intrinsic properties, chiefly the reliability and validity, of the assessment instruments themselves. Our review of the CEQ has often centred on the instrument itself, despite important broader issues (such as lack of learning opportunities and variable training experiences) being raised throughout the consultation phase. A programmatic perspective on the CEQ would instead judge the strength and value of the assessment program by the quality and implementation of the programme as a whole.

In programmatic assessment, a variety of well-chosen and well-developed assessment tools, both written and workplace-based, are implemented in low, medium and high stakes contexts as appropriate. Such a program of assessment thus provides adequate sampling across the breadth and depth of the curriculum, as well as maintaining a focus on the value of meaningful feedback alongside the necessary certification function of assessment systems. This 'dual purposing' is a key principle of programmatic assessment: individual assessments provide targeted feedback for learning, while high-stakes

decisions can be made based on collating the results of multiple assessments (van der Vleuten et al., 2012).

The strength of this approach is that there are multiple opportunities for gaps in learning to be identified, and when combined with an assessment culture which recognises and supports the important formative role of assessment information, allows remedial work to be instigated early and supportively. A programmatic approach should mean that a summative judgment of trainee underperformance at a decision point of the training program should not come as a surprise to either the candidate or the supervisor of training. Multiple data points and opportunities for focussed and supportive feedback and discussion, should identify that a trainee has been struggling to reach the required standard at several stages of the training program.

5.2.3 The need for cultural change

In programmatic assessment, the distinction between training and assessment is deliberately blurred. Some tasks may focus more on their learning function, but still generate assessment data for review from a low-stakes context, while other tasks may focus more on a certification or hurdle function – where the purpose of the assessment is to demonstrate mastery in a high-stakes context. But all tasks that generate assessment information are valuable. This approach is particularly sensitive to implementation challenges, as discrepancies can arise in the perceived stakes and purposes of assessments (Kinnear et al., 2021). Even when learners do theoretically appreciate the breaking of the formative/summative dichotomy into a continuum of stakes, it is hard to shake traditional conceptions that all assessment counts, and everything can begin to feel summative (Pearce, 2022). This only adds to the perceived burden of assessments.

This is where the importance of the supervisor-trainee relationship is especially relevant. Supervisors must be willing to give genuine and potentially negative evaluations, and trainees must see workplace-based assessments as genuine learning opportunities rather than ‘impression management’. Where this does not occur, relatively low-stakes and genuinely formative assessments can be perceived as sequential high-stakes summative assessments and opportunities for trainees to reveal and discuss their own concerns and weaknesses can be lost. Programmatic assessment recognises the important summative role of assessment for progress decisions but seeks to base such progress decisions on rich and cumulative data, not on high-stakes single-point assessments such as examinations. A programmatic approach requires a defensible method for reviewing and evaluating the accumulated data, something normally beyond the practical capacity of single supervisors of training. Instead, programmatic assessment is generally implemented through a combination of the portfolio assessment format (typically learner-led but supported by a faculty mentor), and evaluation by committee (commonly referred to as a ‘progress’ or ‘clinical competence’ committee). These features – use of portfolios, learner-led, mentoring, and evaluation by committee – are crucial elements of the programmatic approach (Driessen et al., 2013; van der Vleuten et al., 2015). The portfolio method facilitates the collation of cumulative assessment data which may then be used by the learner as part of a self-driven approach to learning and identification of learning gaps, supported by an experienced mentor. From a certification perspective, the portfolio also facilitates access to relevant assessment data for consideration by the relevant committee.

Importantly, the use of a progress committee removes the final pass/fail decision from the supervisor, shifting this important decision to a selected group of assessors informed by all available data on the trainee’s performance (including supervisors’ own reports). As such, it can act as a potential counteraction to the ‘failure to fail’ phenomenon (Daly et al., 2022), not only by removing the final

summative decision from the typically close working relationship developed between trainee and supervisor, but also by sharing the decision-making process and evaluation of assessment data between multiple judges. This is not to imply that the evaluative process is necessarily straightforward in a committee context either; indeed, several researchers have noted the challenges and uncertainties that progress committees must work through to arrive at decisions about trainee competence (Chahine et al., 2017; Pack et al., 2019). However, the availability of multiple perspectives (another important example of sampling in assessment) and the opportunity to review and evaluate relevant data through collective and 'effortful discussion' (Pack et al., 2019) support the value of progress committees, alongside programmatic approaches to assessment.

Finally, this approach requires well-planned, robust systems of collecting and collating the data generated by multiple assessments across a training program, these days generally achieved through electronic platforms, suited to the nature of the training program and assessment context (van der Vleuten et al., 2015; George et al., 2020; Weller et al., 2021). It should be noted that the value of a suitable electronic platform is more than just a way of minimising the real administrative burden of programmatic assessment with its focus on frequent assessment and feedback for learning (Ott et al., 2022). It also offers many affordances, such as ready access via mobile phones, voice recognition and automated transcription for recording feedback, prompts and reminders to complete assessment requirements, and automatic capture of patient data, all of which can be used to enhance the value of assessment data for both trainees and supervisors (Weller et al., 2021).

5.3 PROPOSED ALTERNATIVES

We have reached the point in this report where we recommend an alternative assessment format to assess the skills that the CEQ aims to assess. But we need to stress that we are not simply recommending a new assessment instrument as some kind of 'silver bullet' to solve the CEQ conundrum. Drawing on the comments above relating to a programmatic approach to assessment, we strongly recommend that in retiring the CEQ, the intended construct (see construct 2 earlier in this report) needs to become the focus of a range of reconstituted learning *and* assessment activities. This should not be interpreted as simply replacing the CEQ with a new examination. It should be thought of as a broader philosophical shift to both better assess this construct and better facilitate its learning acquisition – embracing this 'dual purposing' of programmatic assessment. None of these activities are unchangeable or fixed – they should be read as options for consideration which will obviously require further conceptualisation, design and development if they are to be implemented.

5.3.1 A structured oral assessment task

We propose a new structured oral assessment task to assess nuanced thinking that embraces multiple perspectives in a psychiatric context. A bespoke name and acronym could be developed, but for simplicity, we will refer to this task as a Structured Oral Assessment (SOA).

- The SOA will be delivered summatively and will be a hurdle requirement.
- It will be facilitated virtually (for example, via Zoom).
- It will be overseen by a dedicated, centralised new SOA committee (likely drawing on the expertise from the existing CEQ committee) tasked with developing the cases, and ensuring consistency of assessment.

- The SOA will include a range of prompts relating to historical, philosophical, ethical, and socio-cultural aspects of psychiatry. The material will be on contested psychiatric issues similar to the current CEQ, rather than clinical issues.
- The SOA committee will develop cases with vignettes, along with specific marking rubrics for each case.
- There will be a range of cases given (suggested three) where the candidates will be presented with a single vignette relating to a contested issue in psychiatry for each component of the assessment.
- The candidates could be given several minutes of thinking time for each case (where they could also take notes), and then a (suggested) 20-30-minute oral assessment will be conducted. The whole SOA will thus take approximately 60 – 90 minutes.
- Two assessors will follow a series of structured questions for each case, but they will be encouraged to ask probing questions that take advantage of the interactive format.
- Both assessors will mark the candidate (blind) using the marking rubric that will be mapped to RANZCP Learning Outcomes and the Fellowship Competencies.
- An example of an case/vignette with a proposed holistic marking rubric is given for illustrative purposes only in Appendix A. This example should be considered a first draft both in terms of content and focus, and as we mention later, trailing and calibration work will be needed to finesse the design and delivery.

This proposal can be thought of as analogous to a Multiple Mini Interview (MMI, Eva et al., 2004) or virtual Objective Structured Clinical Examination (OSCE) format but delivered in a less formal manner. We would want to deliberately downplay comparisons to a high-stakes OSCE, and instead present this assessment format as similar to a medium-stakes Case-Based Discussion (CBD, Sutherland et al., 2019). These formats both involve interaction between the candidate and the assessor. We see this as a strength of the proposed SOA format, where responses can be clarified and probed in real-time (unlike essay writing), and a dialogue can be facilitated around a topic. The literature is supportive of the value of oral assessment formats, and there now exists guidelines for assessors to ensure consistency and validity in their instantiation (Pearce & Chiavaroli, 2020).

Although the sampling and thus reliability of the SOA could be improved with more cases, this needs to be balanced against acceptability and feasibility of the assessment implementation. For this reason, we proposed keeping the number of cases low (approximately three) and seeing this as a trade-off against the formative assessment practice tasks via the corresponding structured learning activities (discussed shortly in Section 5.3.3).

The implementation of the SOA should remain summative as a hurdle requirement. We acknowledge that assessment drives learning, and higher-stakes assessments need to be aligned with the intended outcomes of a training program. Maintaining a summative implementation will encourage trainees and SIMGs to engage more seriously with the relevant content than they may otherwise if the assessment were purely a workplace-based task. To pass the hurdle requirement, candidates would likely need to be rated “at standard” on all the relevant domains, unless specific concession rules are developed.

The success and efficacy of the SOA would depend upon a dedicated panel of assessors (in some form of SOA committee) who were committed to the intent and purpose of the SOA, and were prepared to both develop cases with marking rubrics, and deliver them periodically to candidates. There are considerations here around workloads on existing examiners, but capitalising on the expertise of the

current CEQ team to champion this new format and drive the quality development of the content should be considered.

If the SOA was delivered in the workplace, the SOA could theoretically be delivered at any time during Stage 3, when a candidate believes they are prepared. But for logistical reasons, the assessment could be delivered periodically at set times throughout the year. An alternative centrally delivered option for a summative SOA would likely be delivered at a similar time to the current CEQ. The SOA Committee would, over time, draw on a bank of established vignettes, to ensure wide sampling of topics.

5.3.2 Corresponding structured learning activities

The proposed summative/hurdle requirement of the SOA is only one half of our solution. During our consultations it was clear that the opportunities for trainees to access teaching and learning opportunities for the skills underlying the CEQ during the training program were insufficient, variable or absent. Stakeholders identified a lack of formal teaching (such as classes on ethics and the history of psychiatry), the insufficiency of teaching resources provided by the College on the CEQ format, and variable learning opportunities in the workplace that often depended on individual supervisors.

In response to this, we propose a series of Structured Learning Activities (SLAs) relating to the construct in question, to better facilitate the acquisition of these skills during training. While the SOAs remain a summative hurdle, managed centrally by a dedicated assessment committee to maintain performance standards, standardisation and oversight, the SLAs can in practice become less formal and less standardised to maximise their learning function. However, we call these 'corresponding' SLAs because the assessment format can in fact be identical to the summative SOAs. That is, the SLAs will encourage the development of these important skills as practice tasks to help trainees and SIMGs prepare for their hurdle requirement.

- The SLAs will utilise the same format as the SOA but they will be delivered formatively, in a low-stakes context to focus on the learning function of the tasks.
- SLAs can be facilitated virtually (for example, via Zoom), but could be delivered in-person with supervisors or even more senior trainees or recently graduated Fellows.
- The SOA committee will need to prepare and release a range of learning resources and sample cases/vignettes to provide support for candidates and supervisors. There will need to be a long list of topics (vetted by the College via the committee), with lists of suggested readings for the topics.
- One assessor can deliver the SLA, by following a series of structured questions for each case, and they will be encouraged to ask probing questions that take advantage of the interactive format.
- The assessor will mark the candidate using the marking rubric and record this in an e-Portfolio. These ratings can be monitored over time, and learning progressions should be readily identifiable. This relates to a broader issue of how to implement longitudinal assessment across a whole program.
- These activities are meant to be periodic, and structured, so there should be a formal requirement to do a certain number over a certain period. In order to qualify for an SOA sitting, there would need to be evidence of completing the SLAs over time.

The specific number and period requirements can be determined later, but we suggest set rules are essential. Otherwise the SLAs will likely be done in a haphazard way and become a 'tick-box' exercise.

Mandating some kind of ongoing requirement will encourage this activity more formally, but also provide assessment evidence of learning over time. The establishment of an expert group of SOA Assessors could also reduce the workload burden, and have the added benefit of having the assessors being one step removed from the supervision context. Where this does not occur, relatively low-stakes and genuinely formative assessments can be perceived as sequential high-stakes summative assessments and opportunities for trainees to reveal and discuss their own concerns and weaknesses can be lost. This may enhance the provision of high-quality feedback that is constructive and meaningful.

Again, this is a philosophical shift into thinking about assessment. The aim is to encourage trainees and SIMGs to engage with broader perspectives on psychiatric practice through different lenses. The CEQ was a summative examination designed to determine who was at standard to progress through training. Its function was not explicitly to facilitate learning, although the personal preparation required to succeed in the CEQ demanded some learning. In this new broader design, the hope is that trainee psychiatrists will be encouraged to build their knowledge and understanding of contested issues in psychiatry from multiple perspectives, and the development of these skills will be explicitly fostered through structured tasks that provide diagnostic feedback. The SLAs will act as signposts of gradual increasing competence, so that when a candidate is deemed to be at standard over a period of time, the summative assessment can be delivered to 'confirm' that they are indeed at standard. Through a programmatic lens, this should reduce the stress of the point-in-time CEQ implementation where it is 'all or nothing'.

5.3.3 The provision of a written submission

A further option, if the College wants to maintain a written component to the assessment of this construct, would be to include a written submission as part of the same assessment (see Option 2 in the Executive Summary). Candidates could submit a written piece in response to a prompt from a topic list. This could take the form of an essay but allow the candidate some research and writing time. The piece could encourage reflection from the candidate on the issue at hand, and in a way, could mirror the existing CEQ in some respects.

However, if this were implemented, we would strongly recommend that the written submission becomes the focus of a separate 'station' as part of the oral assessment. The purpose of this additional station would be to allow the assessors to discuss with candidates the submitted content, to ensure that it was the candidates' own work, and to ask some further probing questions in relation to the topic. The written submission could thus become a starting point for more oral assessment and could be conceptualised as similar to an 'oral defence', which is common in some doctoral dissertation processes around the world. The SOA could thus comprise four cases (the three mentioned above and the oral defence of the written submission), or could be reduced to three overall (two novel cases, and one defence of the written submission).

An oral defence of the submission would facilitate a summative assessment of written communication but would have the added benefit of minimising the risk of candidate cheating. It should be made clear to candidates that the work must be their own, and if the assessors are concerned about the authenticity of the work, they will be marked down. In the context of the recent proliferation of generative artificial intelligence (AI) tools, such as ChatGPT, the College should assume that candidates will likely use all the tools available to them to prepare a submission. The College could choose to embrace this fact and interpret the submission as likely to be 'assisted' by AI; with the follow-up oral defence becoming an 'unassisted' assessment moment (Pearce & Chiavaroli, 2023).

The addition of the written submission would increase the preparation and assessment burden on candidates, but it would be a more effective means of assessing the construct the CEQ is currently

meant to be assessing. So in its place, the burden would be equal. There would be increased work for the SOA committee, who would have to develop lists of topics for the written component, and also read and mark the written submissions for each candidate prior to delivering the oral assessment. Nevertheless, this option would maintain the written component that would be lost when the CEQ is retired. Alternatively, the assessment of written skills that would no longer be a part of a wholly oral assessment might be considered as part of a redesigned selection process.

A summary of possible alternatives to the CEQ is provided in Table 1 in the Executive Summary.

6. CONCLUSIONS & RECOMMENDATIONS

From a broad perspective, the CEQ is but one small part of the whole RANZCP program of assessments. Yet for such a small component, it is a complex assessment instrument that raises numerous and multifaceted issues related to learning, training and the broader context of psychiatry.

It is fitting then, that our proposed assessment reform is not simplistic, and deliberately blurs the distinction between training and assessment. We need to stress that we are not simply recommending a new assessment instrument as some kind of ‘silver bullet’ to solve the CEQ conundrum. We have drawn on approaches from programmatic assessment in our recommendations, and suggested that in retiring the CEQ, the intended construct could become the focus of a range of reconstituted learning and assessment activities. This should not be interpreted as simply replacing the CEQ with a new examination. It should be thought of as a broader philosophical shift to both better assess this construct and better facilitate its learning acquisition – embracing this ‘dual purposing’ of programmatic assessment.

The aim is to encourage trainees and SIMGs to more effectively engage with the history and philosophy of psychiatry, and broader perspectives on psychiatric practice through different lenses. Although there is a broader option of exploring avenues for more heavily weighting the assessment of these skills during selection into the training program, it seems more pragmatic to encourage the acquisition of such skills during training. The CEQ is a summative examination designed to determine who is at standard to progress through training. Its function was not explicitly to facilitate learning, although the personal preparation required to succeed in the CEQ demanded some learning. In this new broader design, our hope is that trainee psychiatrists will be encouraged to build their knowledge and understanding of contested issues in psychiatry from multiple perspectives, and the development of these skills will be explicitly fostered through structured tasks that provide meaningful feedback.

We openly acknowledge that our proposed reform will not satisfy all stakeholders. The diverse views represented across a range of stakeholder consultations showed that assessment reform based on stakeholder perceptions alone was impossible. We hope that this detailed and considered report will go some way to alleviating concerns and we welcome feedback and further discussion.

6.1 COMMENDATIONS

Firstly, we would like to make several commendations in relation to the intent of the CEQ and the commitment of all stakeholders.

- The construct that the CEQ is designed to assess appears to add value to psychiatric training. Despite our finding that the CEQ in its current form is not fit-for-purpose, we see the value in its intention, especially with respect to the attempt to assess the non-medical expert Fellowship Competencies in a summative fashion (which in turn drives candidate learning).

- Despite the varied perspectives throughout this review in relation to the CEQ, all stakeholders were highly engaged in providing feedback about this form of assessment. In this sense, there was a clear and strong commitment to improving psychiatric training and a desire to continually improve the assessment of learners.

6.2 RECOMMENDATIONS

Our recommendations are presented in terms of their urgency. Below each recommendation, we have provided some elaboration and proposed actions.

1. Consider the range of findings outlined in this report, and specifically explore the acceptability and feasibility of our proposed SOA/SLA options.

- Seek feedback on the findings and recommendations of this report from relevant RANZCP stakeholders, provide a forum for clarifying issues, and work towards building consensus.
- Seek feedback from stakeholders on the four options proposed by ACER as possible alternatives to the current CEQ that comprise different combinations of Structured Learning Activities (SLAs) and a hurdle Structured Oral Assessment (SOA).
- Consider the strengths and weaknesses of the alternative implementation options.
- We recognise that each of the proposed options may include aspects that are challenging to operationalise due to a range of factors. Thus, for any preferred option a partial or staged implementation should be considered (for instance perhaps rolling out the SLAs in advance of any change to the summative assessment component)

2. Capitalise on the existing CEQ committee to lead a new SOA/SLA Committee tasked with conceptualising, developing and implementing the new suite of SOA/SLA assessment/learning activities.

- Engage existing CEQ examiners as part of this process and seek to leverage their expertise in reforming this new suite of assessment/learning activities.

3. Ensure these new SLA/SOA tasks are clearly linked to the curriculum and make sure the learning outcomes being assessed are relevant and clear.

- There is a current gap between the skills that are meant to be assessed in the CEQ construct and the articulation of these skills as Learning Outcomes in the training program.
- Once the new Learning Outcomes are finalised, ensure that the SOA/SLA solution adequately assesses these skills, and explicitly blueprint SOA/SLA tasks to specific Learning Outcomes.

4. Trial some initial draft SOA/SLAs to inform the finalisation of the assessment requirements.

- Trialling is an important part of the assessment development process. This can be done simply. For examples, once several drafts have been finalised, seek candidates to trial the cases/vignettes in controlled scenarios. The candidates could be psychiatrists who have recently gained RANZCP Fellowship, or even trainees and SIMGs who have recently passed the CEQ. Ask for feedback from participants on all aspects of the assessment process and collate the feedback for evaluation by the SOA/SLA Committee.

5. Finalise and publish the structure of the chosen SOA/SLA process.

- This should include clarity around timing of the assessment, number of cases/vignettes, passing requirements for the hurdle SOA, number and timing requirements of the SLAs, etc. These details need to be communicated prior to the implementation of the reform solution.

- The SOA/SLA Committee may decide to release new sets of vignettes every 6 months, or recommend specific topics, for instance. Details such as these should be worked-through and arrived at by the Committee.

6. *Release a suite of learning resources and support material, including a list of cases/vignettes to support learners in their preparation for the SOA.*

- The SOA/SLA committee will need to prepare and release a range of learning resources and sample cases/vignettes to provide support for candidates and supervisors. There will need to be a long list of topics (vetted by the College via the committee), with lists of suggested readings for the topics.

7. *Consider establishing an Expert SOA Assessors Group.*

- This group of external assessors could support trainees/SIMGs in their preparation and during the SLA and serve in a mentoring capacity to support the development of the relevant skills. Such assessors would be a step-removed from the supervisor context and may be more effective in providing constructive feedback. It would also reduce the burden on supervisors (particularly for Options 1 and 2 where SOAs are not centrally delivered) and draw on the collective expertise of a distributed group of experts who were interested in the SOAs.
- This group could also deliver the summative SOAs and take ownership of the assessment. Some may sit on the SOA/SLA Committee as well, but we envisage the need for a larger pool of centralised assessment experts to reduce the collective workload.

8. *Develop guidelines for SOA Assessors and Supervisors undertaking SLAs, and provide training.*

- The guidelines should outline the purpose of the assessment and learning activities and provide guidance on acceptable forms of prompting and candidate interaction. The SOA Assessors would also need to be briefed and trained periodically in appropriate conduct, and potentially engage in calibration sessions.

9. *Retire the CEQ and replace with the acceptable and feasible options relating to SOA/SLAs.*

- This should only be done once the SOA/SLA Committee has developed sufficient support material and it is feasible to transition to the new assessment/learning activities.

10. *Evaluate the SOA/SLA solution after its initial implementation and commit to continuous improvement of the process.*

- Routine evaluation is good practice, but in this case we strongly recommend evaluating the quality of the SOA/SLA model and the chosen implementation format after the initial roll-out and perhaps every 12 months. If the solution is only partially adopted, the evaluation should review the efficacy of the partial implementation and the need for further adaptations.

11. *Encourage trainees/SIMGs to engage deeply with the history and philosophy of psychiatry and the broad range of social, cultural, ethical and economic factors influencing psychiatric practice.*

- This could be implemented through a range of strategies (to supplement the learning that would occur during the SLAs), such as: developing College run courses in the history and philosophy of psychiatry; hosting lecture series and webinars by expert researchers; developing webinars to teach trainees/SIMGs; or providing book lists and library catalogues on an array of topics.

7. APPENDIX A – ILLUSTRATIVE SAMPLES

SOA/SLA Illustrative Example

Note: The following is an example of how we initially envisage a single case/vignette could look.

The text in the box below could be first presented to the candidate. The assessors would then ask the structured questions and use the relevant prompts where appropriate.

In this Structured Oral Assessment, you will be considering the history and use of electroconvulsive therapy (ECT) from different points of view.

“Critics of ECT portray the treatment as a form of medical abuse, and depictions in the media often show ECT in a critical light. Yet many psychiatrists, and patients, consider ECT to be a safe and effective treatment for some conditions, such as severe depression and bipolar disorder.

ECT was invented in the 1930s as a safer, more humane and less frightening way of inducing seizures than Metrazol. However, in the 1950s ECT was used not just to benefit patients but as a threat to difficult patients, to maintain order on the wards. ECT in the 1950s could be traumatizing and dangerous to the patient, but the invention of modified ECT in modern times, has reduced the danger to the patient significantly.”

Q1. What is your view of criticisms of ECT?

Marking note: candidates are to be judged on their critical reasoning, not whether they hold the ‘right view’.

Prompts (if required):

- Are the criticisms backed up by evidence?
- Are there any landmark studies that demonstrate limitations of ECT?
- Can you cite any landmark studies or other evidence that demonstrate limitations of ECT?

Q2. What are some of the advantages of ECT?

Prompts (if required):

- Are these advantages backed up by evidence?
- Are there any landmark studies that demonstrate that ECT can be a safe and effective treatment for some conditions?
- Can you name any landmark studies that demonstrate that ECT can be a safe and effective treatment for some conditions?

Q3. Are there current ECT practices that are safe for patients?

Prompts (if required):

- How are these safer to patients than in previous time?
- Should ECT have been delivered to patients in the 1950s when these modifications did not exist?

Q4. The use of ECT as a threat to difficult patients in the 1950s raises questions around the psychiatrist – patient relationship. What is your view on this relationship in regards to the ethical use of ECT?

Prompts (if required):

- How was the relationship different historically and in today's context?
- How would you describe the psychiatrist-patient relationship?
- Does this practice in the 1950s of using ECT as a threat to difficult patients constitute ethical use of ECT? Why?

Q5. Historically, ECT was used in some cases as a 'treatment' for homosexuality. This practice has been discontinued. What was the historical cultural context around psychiatry at the time?

Prompts:

- What role did psychiatrists play to shift perspectives and end this practice?
- How does this relate to the role of the psychiatrist as a health advocate?

Potential marking rubric for this example

	Falls well below the standard	Falls below the standard	Meets the standard	Exceeds the standard
Fellowship Competency 1: Communicator	• The use of volume, pace, grammar or vocabulary significantly impedes communication.	• The use of volume, pace, grammar and vocabulary fails to adequately express ideas clearly and coherently.	• The use of volume, pace, grammar and vocabulary are acceptable to express ideas clearly and coherently.	• The use of volume, pace, grammar and vocabulary are sophisticated.
Fellowship Competency 3: Medical Expert, Communicator, Scholar	• Struggles to demonstrate an understanding of the advantages, limitations and/or criticisms of ECT. • Provides insufficient evidence to substantiate claims made. • Demonstrates no evidence of logical argument or critical reasoning: points are random or unconnected or simply listed.	• Demonstrates superficial understanding of the advantages, effectiveness, limitations and/or criticisms of ECT • Provides evidence but with gaps. • Provides a weak attempt at supporting the assertions OR there is only one argument OR the arguments are not well linked.	• Demonstrates a deep understanding of the advantages, effectiveness, limitations and criticisms of ECT whilst providing at least one specific example. • Provides a solid evidence-base to substantiate claims made. • Demonstrates a logical argument.	• Demonstrates a deep understanding of the advantages, effectiveness, limitations and criticisms of ECT whilst providing specific examples. • Provides a substantial evidence-base to substantiate claims made. • Demonstrates a logical and sophisticated argument.
Fellowship Competency 5: Medical Expert, Health Advocate, Professional	• Limits themselves inappropriately rigidly to the medical model OR does not demonstrate cultural awareness or sensitivity where this was clearly required OR fails to	• Touches on the broader models of health and illness, cultural awareness or sensitivity and relevant cultural/historical context but their ideas lack depth or breadth or are	• Demonstrates an acceptable level of cultural sensitivity and/or historical context and/or broader models of health and illness and/or the role of psychiatrist as	• Demonstrates a superior level of awareness and knowledge of cultural sensitivity and/or historical context and/or broader models of health and illness and/or the role of psychiatrist as

	demonstrate an appropriate awareness of a relevant cultural/historical context • Demonstrates a lack of proficiency in health advocacy in a professional setting.	inaccurate or irrelevant to the use of ECT • Demonstrates little reflection on historical actions and lessons learned.	advocate relevant to the use of ECT. • Demonstrates reflection on historical actions and lessons learned.	advocate relevant to the use of ECT. • Demonstrates reflection on historical actions and lessons learned, and how the past has impacted upon the perceptions of ECT and psychiatry today.
Fellowship Competency 6: Professional	• Demonstrates a lack of proficiency in professionalism and/or unprofessional behaviour. • Fails to address ethical issues.	• Demonstrates some examples of professionalism in a psychiatry context. • Raises ethical issues that are not relevant or are simply listed without elaboration or are described incorrectly or so unclearly as to cloud the meaning	• Demonstrates clear and consistent examples of professionalism in a psychiatry context. • Demonstrates an appropriate awareness of relevant ethical issues.	• Demonstrates exceptional examples of professionalism in a psychiatry context. • Demonstrates a superior level of knowledge or awareness of relevant ethical issues.
Fellowship Competency 7: Medical Expert, Collaborator	• Demonstrates a poor connection between the safety and effectiveness aspects of ECT today and historically • Provides insufficient evidence to substantiate claims made. • Fails to consider patient-centred care, carers, and/or recovery principles where these are relevant OR merely mentions them.	• Demonstrates a sound connection between the safety and effectiveness of some of the aspects of ECT today and historically • Provides evidence but with gaps. • Mentions patient-centred care, carers, and/or recovery principles where these are relevant but does not demonstrate an accurate understanding of them or is unable to do so clearly.	• Demonstrates a sound connection between the safety and effectiveness aspects of ECT today and historically, including at least one specific example of how modified ECT is safer. • Provides a solid evidence-base to substantiate claims made. • Demonstrates understanding of patient-centred care, the recovery model in psychiatry, and the role of carers.	• Demonstrates a mastery in coupling the safety and effectiveness aspects of ECT today and historically, including specific examples of how modified ECT is safer. • Provides a substantial evidence-base to substantiate claims made. • Demonstrates a superior depth or breadth of understanding of patient-centred care, the recovery model in psychiatry, and the role.

8. APPENDIX B – FINDINGS FROM DESKTOP MEQ REVIEW

The Modified Essay Questions (MEQ) examination is designed to assess a candidate's critical thinking about clinical practice and their ability to apply clinical knowledge. The examination is paper-based and consists of 4-6 modified essay questions worth a total of 125 marks. Candidates have 150 minutes to complete the MEQ Examination, including reading time.

The MEQ was de-coupled from the CEQ in 2021 following a recommendation arising from the ACER review of the RANZCP examinations. The pass rates for the MEQ since that time are shown in Table 4.

Table 4: Pass rates for MEQ Sitzings since 2021.

MEQ Sitting	Pass rate
Feb, 2023	60.4%
Aug, 2022	54.8%
Feb, 2022	62.0%
Aug, 2021	48.0%

ACER was asked to review the MEQ examination with a view to quality improvement and to evaluate the probable cause of low MEQ pass rates. The format of the exam, and advice about the suitability of offering the MEQ online will also be discussed. Some stakeholders also discussed issues relating to the MEQ during the consultations. Feedback from the consultations will also be included in the review.

The review of the MEQ will cover the following:

- quality of the questions and marking guides from an assessment perspective
- the marking processes and their implementation
- the standard setting process and outcomes
- support for candidates including candidate feedback
- the suitability of an online format

Quality of the Questions and Marking Guides

Questions on the MEQ examination are delivered in the form of a vignette (Figure 3). Each question is blueprinted to patient age group (where applicable), relevant topics, and the syllabus areas. The questions based on the vignette can be quite open-ended. For example, consider Question 5.1 of the 2022 Sitting 1 of the MEQ (Figure 3):

Describe (list and justify) the factors that may have contributed to Stan's poor engagement with mental health services in the past. Please note: a list with no explanation will not receive any marks. (10 marks).

The examiners should be commended for specifying to candidates that they must provide an explanation of the factors, without which they will not be awarded any marks. From the marking guide, there are up to 4 acceptable factors for Stan's poor engagement (Figure 4). Each factor is worth a different number of marks e.g. discussion around family factors is worth up to 2 marks while discussion around individual/illness factors is worth up to 4 marks depending on the number of sub-factors discussed.

Modified Essay 5

Each question within this modified essay will be marked by a different examiner. The examiner marking this question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in a community mental health centre. A general practitioner refers Stan, a 37-year old unemployed electrician to you.

Stan has a 20-year history of obsessive compulsive disorder. He is unable to leave the house because of checking rituals. He lives with his elderly parents who shop and cook for him. He has a long history of intermittent contact with mental health services but has never attended for longer than a few months at a time. He has tried many different medications but has stopped them because he could not tolerate the side-effects.

Question 5.1

Describe (list and explain) the factors that may have contributed to Stan's poor engagement with mental health services in the past.

Please note: a list with no explanation will not receive any marks. (10 marks)

Figure 3: MEQ 5, Question 5.1 from Sitting 1, 2022

A.	INDIVIDUAL/ILLNESS FACTORS: - Treatment Pessimism. Pessimistic attitude towards treatment, including stigma, poor insight, stage of change, lack of treatment understanding and previous treatment failures. Can include frustration and hopelessness. - The OCD itself. Severe OCD may have made it difficult to leave the house and attend appointments. This may be related to overwhelming anxiety and the time consuming nature of the rituals. - Comorbidities: psychosis; personality factors, such as passive aggressive, avoidant, paranoid personality traits. substance abuse. - Impact of OCD on Stan's developmental trajectory.	① ① ② ③ ④
B.	FAMILY FACTORS: - Parents tolerate and accommodate OCD symptoms, so no impetus to change. - Family attitudes about mental illness and mental health services. - Parents' lack of awareness of treatment options which could improve his level of functioning and the illness. - Family violence – dynamic of avoidance and fear of Stan becoming violent. - Stigma – self, parental, community. - Relational dynamics between Stan and his parents may contribute to his poor engagement by his acting out (punishing his parents, for example). - Parental health including frailty.	① ① ②
C.	TREATMENT FACTORS: - Inadequate or inappropriate treatment e.g. benzodiazepines or only one treatment modality e.g. never offered CBT. - Medication side-effects, poor understanding of expected response times and duration of treatment. - Failure to address comorbidities, e.g. substance dependence, depression, other anxiety disorders. - Failure to address systemic barriers to treatment e.g. failure to address possible parental factors. - ERP/CBT may cause Stan to become distressed or anxious.	① ① ② ③
D.	MENTAL HEALTH SERVICES FACTORS: - Difficulty following up an ambivalent patient. - Service capacity and lower prioritisation as Stan may be perceived as not being at high risk. - Lack of staff trained in relevant psychotherapy e.g. CBT. - Failure to develop a therapeutic alliance e.g. through failure to personalise treatment. - Lack of continuity of treatment staff. - Poor attunement between staff and Stan. - Accessibility – distance, transport.	① ① ② ③

Figure 3: Marking guide for MEQ 5.1 from Sitting 1, 2022

Family factors in Question 5.1 is worth 2 marks maximum, however, candidates are not made aware of this mark breakdown. Candidates could potentially spend a substantial amount of time discussing family factors and spend little to no time discussing mental health service factors. During the CEQ consultations, one stakeholder described how the open-ended nature of the MEQ questions were difficult as they believed that it was unclear how much detail was expected in any given response.

Candidates may perform better at this question with more scaffolding e.g. Question 5.1 could be written as:

Describe (list and justify) the factors that may have contributed to Stan's poor engagement with mental health services in the past. Consider individual, family, treatment and service factors. Please note: a list with no explanation will not receive any marks. (10 marks).

By asking candidates to consider individual factors, candidates should be oriented to discuss Stan's illness, attitude and previous treatments. By also mentioning family, treatment and service factors, candidates should be aware that they need to demonstrate their understanding of how these factors can also effect Stan's engagement with mental health services in the past. Additionally, the question could be broken down further to specify to candidates the maximum number of marks for different factors.

Describe (list and justify) the factors that may have contributed to Stan's poor engagement with mental health services in the past. Consider individual (4 marks), family (2 marks), treatment (3 marks) and service factors (3 marks). Please note: a list with no explanation will not receive any marks. (Total of 12 marks).

The marking rubric for the MEQ tends to be in the form of potential model answers as shown in Figure 4, with little to no guidance to examiners around the breakdown of marks e.g. Treatment factors can be worth up to 3 marks BUT how much is needed for 1 mark? What type of response would be sufficient for a score of 2 marks? Some questions have the same number of bullet points as marks, so without explicit guidance, examiners may assume they should apply one mark per bullet point e.g. Question 5.1 Domain A, in Figure 4.

Team leaders do provide some guidance to examiners through the marking process to ensure consistency, but by refining the marking guides to make the mark breakdown clearer to examiners this should reduce the need for team leaders to provide as much guidance.

Also, some questions are worth 8 marks but candidates can score higher than the designated marks, e.g. see Question 5.2 where candidates can receive up to 11 marks (Figure 5). This misalignment between the question and the marking guide is intentional but it is unclear how this is communicated to candidates.

Question 5.2 Describe (list and explain) the key features of your psychological treatment plan for Stan. <i>Please note: a list with no explanation will not receive any marks. (8 marks)</i>		
A.	PSYCHOEDUCATION: - Provide information to the patient and family about the condition in a way that has personal relevance. - Provide information that exposure and response prevention, and cognitive therapy have the best evidence base. - Explain rationale of chosen psychological treatment.	0 1 2
B.	COGNITIVE BEHAVIOUR THERAPY: - Initial assessment and formulation, assess preparedness to change; identify maintaining factors: triggers, avoidance and safety behaviours. - Development of exposure hierarchy. Use of a measurable monitor of change, e.g. Subjective Units of Distress Scale, YBOCS, or other. - Choose goals to work on and set specific homework tasks. - Confront each chosen situation, refrain from engaging in compulsive ritual and stay in situation until anxiety subsides. - Monitor using an appropriate outcome measurement e.g. role of Goal Attainment Scale.	0 1 2 3 4 5
C.	FAMILY INTERVENTION: - Family therapy to support understanding of and responses to enduring patterns. - Negotiate role for parents; practical advice for parents assisting them not to inadvertently do things for him instead of with him e.g. by completing tasks for him. - Identify the dynamics/parental responses which may be reinforcing the illness, e.g. conflict avoidance.	0 1 2
D.	LONG TERM RECOVERY: - Maximising quality of life even in the context of chronic disorder, including vocational and functional rehabilitation. - Learning to live with OCD. - Identifying and encouraging realistic goals. - Note to Markers: mentioning recovery without contextualising or justifying its use here is not acceptable.	0 1 2

Figure 5: Marking guide for MEQ 5.2 from *Sitting 1, 2022*

Marking Processes

Different examiners are assigned to mark each MEQ question, and sometimes different examiners are assigned to sub-questions. There are 4-6 questions in total, and 2 markers per question sub-part. A 3rd marker is also present when discrepancies arise.

From the College's MEQ Standard operating procedures:

Each of the 4 - 5 MEQ's within the Examination paper have a series of 'sub' questions running off them (eg: MEQ 1.1, 1.2, 1.3). Each sub-question could be marked by a different marker, meaning that markers will not see the answers candidates gave to the other sub-questions of the same essay. Markers with that sub-question can only mark what they have been given.

There is a calibration process of marking 5-7 scripts.

Some stakeholders described the MEQ marking rubric as too vague and contended that markers may feel limited by the model answers provided on the marking sheet. This is a typical issue with model answers, as for some types of questions, candidates can provide answers that are equally correct and/or relevant but which deviate from a model answer. In this case, there is no guidance to markers on how to mark these responses.

Having different examiners per sub-question can also prove problematic as the sub-questions all depend on the same vignette. For example, if one sub-part asked candidates to discuss key issues in maintaining a safe environment for a patient and staff, then another sub-part might ask candidates to discuss methods to prevent a patient's behaviour deteriorating. Some candidates may discuss key issues in the first sub-part which are relevant to the second sub-part. Though the paper clearly states that: "each question within this modified essay will be marked by a different examiner" and "the examiner marking this question will not have access to your answers to the other questions" candidates may inadvertently answer the first sub-part with some of the content that should be in the second sub-part, then not repeat this content in the second sub-part. The practice of having different examiners per question sub-part should be carefully considered, as in some cases, the marks may not truly reflect the ability of the candidate.

The risk posed by independent markers for sub questions is heightened if the marking guides for two questions overlap. From the review of the MEQ, this was rarely the case, however there were a few instances between 2021 and 2023 where this overlap did occur.

Standard Setting and Cut-Score Determination

Standard setting is undertaken separately for each MEQ sub-question. The procedure outlined in the College's MEQ Standard operating procedures states that:

Standard setting of the examinations is done by the Committee for Examinations and at a local level to obtain a cross-section of opinion. The Committee for Examinations currently uses the Modified Ebel Method of standard setting.

However, the standard setting approach used in practice appears to be an extended Angoff method. Both the Committee for Examinations (CFE) and the Satellite group consider each question sub-part and give the marks they would expect a minimally competent candidate to receive on the question sub-part. An impact analysis session is held, where the standard setting for each individual sub-part question is reconsidered in light of the data from the examination. The percentage difference of the average score on the question to the score determined by the standard setting (cut-score) is calculated. In these

meetings, after careful deliberation, examiners may decide to reduce their cut-score for a question sub-part. The cut-score is then recalculated for the examination.

The overall cut-score for the examination is determined using the cut-scores of the sub-questions rated by the two groups. There is an error margin of 1 SEM. Any candidate whose score is equal to or above the overall cut-score – 1 SEM is awarded a pass. There is a conceded pass for anyone within 0.5% of the passing standard.

Support for Candidates

Candidates receive support from the College to prepare for the MEQ through a variety of online resources:

- Post-examination reports discussing the individuals' MEQ responses and areas for candidates to improve, along with releasing one MEQ and its marking guide
- Slides from the 2022 MEQ Writtens Congress – *A behind-the-scenes look at the MEQ Examination* which details the purpose of the MEQ, how questions are set, standard setting, how questions are marked, and general tips and strategies
- Slides from the 2021 & 2018 MEQ Writtens Congress
- Podcast on *Preparing for the Modified Essay Questions (MEQ) Examination*

Candidates also receive detailed feedback letters after they sit the MEQ. ACER reviewed the feedback letters received by trainees that passed and failed the MEQ. The feedback on both the Pass and Fail letters is extensive. Both letters provide trainees with their overall result as well as a breakdown of their performance by content area, compared to the mean performance of the cohort (Figure 66). This level of feedback directs trainees to see which content areas require further study before completing another sitting of the examination.

Re: February 2022 MEQ Examination Result – RANZCP ID 1XXX

You have been successful in completing the MEQ Examination.

Please find detailed below your result.

Your MEQ Examination result	Maximum marks
55.2%	125

Given below is your performance by question content.

MEQ Content (February 2022)	Your performance	Mean performance (cohort)	Proportion of marks
Assessment	56.7%	65.9%	9.6%
Basic sciences, medical knowledge	12.5%	24.1%	4.8%
Epidemiology, Diagnosis & Classification and public health	50.0%	66.8%	4.8%
Leadership, governance and relevant legal frameworks	68.8%	60.1%	6.4%
Professional communication and liaison	64.3%	59.8%	5.6%
Psychology, philosophy and psychodynamic principles	80.0%	53.7%	4.0%
Sociocultural awareness	54.7%	59.4%	15.2%
Specific areas of practice - Child and adolescent	50.0%	33.0%	4.8%
Specific areas of practice - Forensic	60.0%	49.6%	1.6%
Specific areas of practice - Indigenous/Maori	66.0%	59.5%	8.0%
Specific Disorders - Anxiety	50.0%	58.2%	3.2%
Specific Disorders - Impulse control/Mood/Organic	40.0%	64.0%	4.0%
Specific Disorders - Sleep	40.6%	38.9%	6.4%
Specific Disorders - Substance Use disorders	100.0%	66.8%	2.4%
Treatments in psychiatry	54.2%	54.0%	19.2%

Figure 6: Extract from a successful candidate's letter from Sitting 1, 2022

The letter for unsuccessful candidates also provide a graph which shows the trainee's overall performance measured against the cut-score (Figure 7). However, the feedback letter does not include any explanation of how the cut-score was derived.

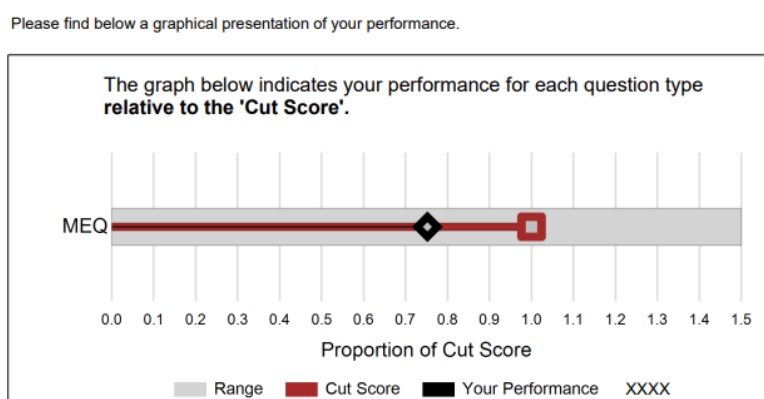


Figure 7: Extract from an unsuccessful candidate's letter from Sitting 1, 2022

A further point raised during the stakeholder consultations related to a lack of question-specific feedback for the MEQ. It was then difficult to know what aspects of a candidate's response were not sufficient and/or correct. The examination reports do provide general feedback for each MEQ for that sitting. The general feedback identifies the skill and/or knowledge being tested, describes what trainees found difficult and provides general advice to the trainee. The report ends with some overall comments about the areas in which trainees performed particularly badly. The examination report also provides the average marks achieved on each question (Figure 8) and clearly identifies the percentage of candidates that passed and failed that particular sitting.

MEQ	Marks worth	Average mark achieved (with SD)
1	29	17.62 (3.4)
2	35	17.88 (3.9)
3	36	14.81 (3.7)
4	25	10.52 (3.3)
		Overall average mark 60.83/48.7%

Figure 8: Average marks achieved for each MEQ in Sitting 2, 2022

Feedback from stakeholders suggested that candidates did not do well at the MEQ because they lack clinical experience. It is recommended in the online resources (written congress presentation for 2022) that candidates should only sit the MEQ once they feel they have sufficient clinical experience. The College website states that a candidate is eligible to apply for the MEQ after completing 18 months of training. However, candidates may take up until 60 months of training to successfully complete the MEQ.

Suitability for an Online Format

The stakeholder consultations suggested that trainees believed that the purpose of the MEQ was clearer than the CEQ because the MEQ was directly relevant to clinical practice. However, it was also noted that the paper-based format of the MEQ and procedural issues in the marking of the MEQ could substantially affect decisions about trainee competence on the MEQ. For instance, should a trainee

incorrectly enter a response against the wrong sub question, the examiner marking that sub question would not be aware as they would not have access to the complete paper.

The paper-based format for the MEQ also assesses aspects of the candidates' proficiency that are unrelated to critical thinking or the candidates' clinical competence. The MEQ format rewards candidates who can handwrite rapidly and legibly under time pressure. In this respect, the criticisms directed at the CEQ and MEQ formats were similar. Although arguably some capacity for legible handwriting and operating under time pressure are features of the work of consultant psychiatrists, these aspects of the psychiatrists practice are not what the MEQ is intended to assess.

An online format for the MEQ could overcome two fundamental issues: 1) relying on candidates placing the correct sub-question number next to their response, and 2) relying on legible handwriting in a time constrained assessment. Adopting an online format may better align to practice, as typing responses and patient reports is more common practice in the workplace. An online format would more closely resemble the skills needed in clinical practice. Though handwriting is important, for note taking, it is not commonly used now for the development of reports.

However, online formats have their own challenges in the form of ensuring the integrity of responses, and often rely on using testing centres or digital proctoring. Many colleges have moved to online formats using a combination of invigilated examinations in testing centres and digital proctoring. Although we recommend moving to a digital format for the MEQ, the College will need to consider whether the examination will be administered in testing centres in central locations, or whether a proctoring option is feasible. During the COVID pandemic, many colleges delivered written examinations via proctoring, and many have continued to do so post-COVID due to reduced costs. However, there are other challenges with proctoring, such as the potential for candidates to cheat during breaks, but also requiring candidates to have a good office set up and reliable internet connection. Home life, noisy children, noise from construction works, and unreliable internet connections, all add other complications to a proctored set-up, which is why test-centres are still considered to be the most reliable and standardised approach to digital testing.

Summary

Overall our review of the MEQ did not uncover any major flaws. In terms of the quality of the questions and marking guides, the questions are generally clear and unambiguous, with marking rubrics that mostly align to the questions. There is also clear blueprinting to topics, age brackets and the curriculum. Further guidance could be given to candidates by stating the number of diagnoses/treatments/management practices that need to be discussed to achieve full marks. More detail could also be given to examiners on the breakdown of marks.

In terms of the marking processes and their implementation, these appeared to be functioning effectively. Calibration after the marking of 5-7 scripts follows best-practice for essay-style questions. Each sub-part question should be carefully considered. If there is a significant risk that some sub-part answers will overlap, the same examiners should be used to mark those sub-part questions. In terms of standard setting, it should be noted that pass-rate variations from cohort to cohort are to be expected. As this is a criterion-reference examination, with criterion-reference standard setting (as it should be), pass rates will vary, and minor variations in the pass rates are best to be interrogated during the impact analysis session.

In terms of support for candidates and candidate feedback, the examination reports include extensive detail, advice to candidates and the release of one of the MEQs with its marking guide. The letters provide detailed feedback to candidates, including a breakdown of how they performed in different content areas. The written congress slides are also informative, and give an excellent overview of the MEQ development, standard setting and final result process.

Recommendations relating to the MEQ

A1. Move the MEQ to an online format.

- In doing so, carefully consider the impacts of a digital proctored solution as well as the feasibility of using testing centres prior to finalising the delivery method. It may be worth exploring the feasibility of both options.

A2. Update the standard setting terminology used in the MEQ marker recruitment and calibration Standard Operating Procedures.

- The current version of the *MEQ marker recruitment and calibration Standard Operating Procedures* states that standard setting is performed using the Ebel methodology. However, the committees appear to be using an extended Angoff method where the score of the minimally competent candidate is estimated for each question sub-part. The Standard Operating Procedures should be updated to reflect the current standard setting process.

A3. Consider providing advice on when to sit the MEQ.

- The range of 18 months to 60 months is quite broad. It could be worth analysing the average number of months a candidate has been in the program when they successfully complete the MEQ and providing some advice on this.

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